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Report No: PAD3539

INTERNATIONAL BANK FOR RECONSTRUCTION AND DEVELOPMENT

PROJECT APPRAISAL DOCUMENT

ON A

PROPOSED LOAN

IN THE AMOUNT OF US\$20 MILLION

TO THE

KINGDOM OF ESWATINI

FOR A

HEALTH SYSTEM STRENGTHENING FOR HUMAN CAPITAL DEVELOPMENT IN ESWATINI PROJECT

May 29, 2020

Health, Nutrition, and Population Global Practice
Africa Region

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CURRENCY EQUIVALENTS

(Exchange Rate Effective April 30, 2020)

Currency Unit = Eswatini Lilangeni (SZL)

SZL 18.3 = US\$1

FISCAL YEAR

April 1 - March 31

ABBREVIATIONS AND ACRONYMS

AIDS	Acquired Immune Deficiency Syndrome
ANC	Antenatal Care
ASA	Analytical and Advisory Services
CBA	Cost-Benefits Analysis
CERC	Contingent Emergency Response Component
CHV	Community Health Volunteer
CMIS	Client Management Information System
CMS	Central Medical Store
CoP	Community of Practice
CPIA	Country Policy and Institutional Assessment
COVID-19	Coronavirus Disease
CPS	Country Partnership Strategy
DP	Development Partner
E&S	Environmental and Social
EHS	Environment, Health and Safety Guideline
eLMIS	Electronic Logistical Management Information System
EMMS	Electronic Maintenance Management System
ESCP	Environmental and Social Commitment Plan
ESF	Environmental and Social Framework
ESMF	Environmental and Social Management Framework
FM	Financial Management
FMS	Financial Management Specialist
GBV	Gender-based Violence
GHG	Greenhouse Gas
GDP	Gross Domestic Product
GOE	Government of Eswatini
GRM	Grievance Redressal Mechanism
GRS	Grievance Redress Services
HCI	Human Capital Index
HCWM	Health Care Waste Management
HIV	Human Immunodeficiency Virus

HMIS	Hospital Management Information System
HRH	Human Resources for Health
IBRD	International Bank for Reconstruction and Development
ICDF	International Cooperation and Development Fund
ICWMP	Infection Control and Waste Management Plan
IFR	Interim Financial Report
IMF	International Monetary Fund
IPF	Investment Project Financing
IT	Information Technology
LMP	Labor Management Procedure
M&E	Monitoring and Evaluation
MCH	Maternal and Child Health
MGH	Mbabane Government Hospital
MICT	Ministry of Information Communication and Technology
MOEPD	Ministry of Economic Planning and Development
MOF	Ministry of Finance
MOH	Ministry of Health
MOPS	Ministry of Public Services
NCD	Noncommunicable Disease
NHSSP	National Health Sector Strategic Plan
OHS	Occupational Health and Safety
PDO	Project Development Objective
PEPFAR	President's Emergency Plan for AIDS Relief
PFMA	Public Financial Management Act
PHC	Primary Health Care
PIU	Project Implementation Unit
PPSD	Project Procurement Strategy for Development
PS	Principal Secretary
QMA	Quality Management Approach
RFMH	Raleigh Fitkin Memorial Hospital
RHM	Rural Health Motivator
RHMT	Regional Health Management Team
RMNCAH	Reproductive, Maternal, Newborn, Child, and Adolescent Health
RPF	Resettlement Policy Framework
SARA	Service Availability and Readiness Assessments
SBCC	Social and Behavioral Change Communication
SEA	Sexual Exploitation and Abuse
SEP	Stakeholder Engagement Plan
STEP	Systematic Tracking of Exchanges in Procurement
TA	Technical Assistance
TB	Tuberculosis
UHC	Universal Health Coverage
UNFPA	United Nations Population Fund
UNICEF	United Nations Children's Fund
WHO	World Health Organization
WHO-PEN	World Health Organization Package of Essential NCD Interventions

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DATASHEET

BASIC INFORMATION

Country(ies)	Project Name	
Eswatini	Health System Strengthening for Human Capital Development in Eswatini	
Project ID	Financing Instrument	Environmental and Social Risk Classification
P168564	Investment Project Financing	Moderate

Financing & Implementation Modalities

☐ Multiphase Programmatic Approach (MPA)	☒ Contingent Emergency Response Component (CERC)
☐ Series of Projects (SOP)	☐ Fragile State(s)
☐ Performance-Based Conditions (PBCs)	☒ Small State(s)
☐ Financial Intermediaries (FI)	☐ Fragile within a non-fragile Country
☐ Project-Based Guarantee	☐ Conflict
☐ Deferred Drawdown	☐ Responding to Natural or Man-made Disaster
☐ Alternate Procurement Arrangements (APA)	

Expected Approval Date	Expected Closing Date
22-Jun-2020	30-Sep-2025

Bank/IFC Collaboration

No

Proposed Development Objective(s)

The project development objective is to improve the coverage and quality of key reproductive, maternal, neonatal, child and adolescent health (RMNCAH), nutrition and NCD services (hypertension and diabetes) in Eswatini.

Components

Component Name	Cost (US\$, millions)
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Component 1: Improve health service delivery to increase the coverage and quality of health services to build human capital	14.50
Component 2. Increase community demand for RMNCAH, nutrition and NCD services	2.00
Component 3. Strengthen the MOH's stewardship capacity to manage essential health and nutrition services and project activities	3.50
Component 4: Contingent Emergency Response Component	0.00

Organizations

Borrower: Kingdom of Eswatini

Implementing Agency: Ministry of Health

PROJECT FINANCING DATA (US\$, Millions)

SUMMARY

Total Project Cost	21.00
Total Financing	21.00
of which IBRD/IDA	20.00
Financing Gap	0.00

DETAILS

World Bank Group Financing

International Bank for Reconstruction and Development (IBRD)	20.00
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Non-World Bank Group Financing

Counterpart Funding	1.00
Borrower/Recipient	1.00

Expected Disbursements (in US\$, Millions)

WB Fiscal Year	2020	2021	2022	2023	2024	2025	2026
Annual	0.00	3.00	5.00	5.00	4.00	2.00	1.00
Cumulative	0.00	3.00	8.00	13.00	17.00	19.00	20.00

INSTITUTIONAL DATA

Practice Area (Lead)

Health, Nutrition & Population

Contributing Practice Areas

Climate Change and Disaster Screening

This operation has been screened for short and long-term climate change and disaster risks

SYSTEMATIC OPERATIONS RISK-RATING TOOL (SORT)

Risk Category	Rating
1. Political and Governance	● Substantial
2. Macroeconomic	● Substantial
3. Sector Strategies and Policies	● Low
4. Technical Design of Project or Program	● Moderate
5. Institutional Capacity for Implementation and Sustainability	● Substantial
6. Fiduciary	● Moderate
7. Environment and Social	● Moderate
8. Stakeholders	● Moderate
9. Other	
10. Overall	● Moderate

COMPLIANCE

Policy

Does the project depart from the CPF in content or in other significant respects?

☐ Yes ☒ No

Does the project require any waivers of Bank policies?

[] Yes [✓] No

Environmental and Social Standards Relevance Given its Context at the Time of Appraisal

E & S Standards	Relevance
Assessment and Management of Environmental and Social Risks and Impacts	Relevant
Stakeholder Engagement and Information Disclosure	Relevant
Labor and Working Conditions	Relevant
Resource Efficiency and Pollution Prevention and Management	Relevant
Community Health and Safety	Relevant
Land Acquisition, Restrictions on Land Use and Involuntary Resettlement	Relevant
Biodiversity Conservation and Sustainable Management of Living Natural Resources	Not Currently Relevant
Indigenous Peoples/Sub-Saharan African Historically Underserved Traditional Local Communities	Not Currently Relevant
Cultural Heritage	Relevant
Financial Intermediaries	Not Currently Relevant

NOTE: For further information regarding the World Bank's due diligence assessment of the Project's potential environmental and social risks and impacts, please refer to the Project's Appraisal Environmental and Social Review Summary (ESRS).

Legal Covenants

Sections and Description

Loan Agreement. Section I.A of Schedule 2.

By no later than one (1) month after the Effective Date, the Borrower shall establish and thereafter maintain, at all times during the implementation of the Project, an inter-ministerial policy advisory group, with a mandate, terms of reference, composition, and resources satisfactory to the Bank, to be responsible for informing cabinet-level decision making and policy formulation. Notwithstanding the aforementioned, said group shall: (a) report to the Secretary to the Borrower's Cabinet; (b) be chaired by the Principal Secretary of the MOH; and (c) consist of representatives from the Central Agencies and the Chief Economist of MOEPD.



Sections and Description

Loan Agreement. Section I.A of Schedule 2.

By no later than one (1) month after the Effective Date, the Borrower shall establish and thereafter maintain, at all times during the implementation of the Project, an inter-ministerial technical committee, with a mandate, terms of reference, composition, and resources satisfactory to the Bank, to be responsible for: (a) guiding Project implementation and operationalization at a technical level; (b) reviewing and discussing technical aspects that require coordination with the Central Agencies and other stakeholders; and (c) preparing reports and/or proposals for the guidance, information and/or decision of the inter-ministerial advisory group referred to in Section I.A.1 above. Notwithstanding the aforementioned, said committee shall: (a) be chaired by the Director of Health Services of MOH; and (b) consist of representatives of the sectoral officers listed in the Project Operations Manual.

Sections and Description

Loan Agreement. Section I.B of Schedule 2.

By no later than one (1) month after the Effective Date, the Borrower shall adopt and thereafter carry out the Project in accordance with the Project Operations Manual referred to in Section I.B of Schedule 2. The Borrower shall carry out the Project in accordance with the a project operations manual ("Project Operations Manual"), which contains detailed guidelines and procedures for the implementation of the Project, including: administration and coordination; monitoring and evaluation; financial, procurement and accounting procedures; social and environmental safeguards; corruption and fraud mitigation measures; roles and responsibilities of various ministries, departments, agencies in the implementation of the Project; and such other arrangements and procedures as shall be required for the effective implementation of the Project. Except as the Bank shall otherwise agree in writing, not amend or waive, or permit to be amended or waived any provision of the Project Operations Manual.

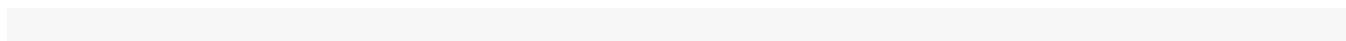
Sections and Description

Loan Agreement. Section I.A of Schedule 2.

By no later than one (1) month after the Effective Date, the Borrower shall establish and thereafter maintain at all times during the implementation of the Project, a Project implementation unit within MOH, with a mandate, terms of reference, composition, and resources satisfactory to the Bank, to be responsible for: (a) managing and coordinating the implementation of the Project; and (b) serving as secretariat to the inter-ministerial policy advisory group referred to in Section I.A.1 above. Notwithstanding the aforementioned, said unit shall: (a) report to the Principal Secretary of MOH; (b) be led by a full-time Project Coordinator; and (c) comprise of full-time MOH staff and technical contracted staff, who shall be assigned and/or hired, and thereafter maintain at all times during the implementation of the Project, in a number and with terms of reference, qualifications and experience satisfactory to the Bank.

Conditions

Type	Description
Disbursement	Loan Agreement. Section III.B.1(b) of Schedule 2. For Emergency Expenditures under Category 2, unless and until the Bank is satisfied, and notified the Borrower of its satisfaction, that all CERC conditions have been met.





I. STRATEGIC CONTEXT

A. Country Context

1. **The Kingdom of Eswatini is a mountainous, landlocked, small open economy in Southern Africa, with four administrative regions.** The population is 1.1 million, and 78 percent live in rural areas. As a 'monarchical democracy', absolute power rests with the monarch. Traditional and parliamentary systems run concurrently. The Kingdom has close economic links to South Africa on which it depends for about 85 percent of its imports and 60 percent of its exports. Eswatini is a member of the Common Monetary Area with Lesotho, Namibia, and South Africa, under which the Eswatini lilangeni (SZL) is pegged at par to the South African rand.

2. **Although classified as a lower-middle-income country (gross national income per capita of US\$2,960), high poverty rates and income inequality (Gini coefficient of 49.3) challenge Eswatini's economic and human development potential.** While the national poverty rate¹ has fallen in recent years (from 63 percent in 2010 to 59 percent in 2017), it remains very high, particularly in rural areas (70 percent) and in two regions (Lubombo [72 percent] and Shiselweni [67 percent]). Eswatini's hunger levels are 'serious';² almost two-thirds of the population are food insecure³ with detrimental impact on pregnant women and children.

3. **Macroeconomic performance has been hampered by severe droughts due to climate change and growing fiscal challenges emanating from rapid growth in the public wage bill combined with volatile Southern African Customs Union transfers.** Gross domestic product (GDP) growth rate fell from 2.4 percent in 2018 to 1.3 percent in 2019. The 2019 contraction caused a high fiscal deficit and cash flow challenges. In 2019, the International Monetary Fund (IMF) urged Eswatini to undertake expenditure rationalization.⁴ With the Government's focus on reducing the structural and fiscal imbalances, the 2020–21 budget depicts a positive outlook with a growth recovery up to 2.9 percent in 2020.⁵

4. **Because of the outbreak of the 2019 novel coronavirus, growth projections have been muted to below 1 percent for 2020.** As the spread of the virus severely affects trade and supply chains and depresses consumption and demand in affected countries, economic growth has been revised down substantially in China, the United States, and the G-20 economies, including South Africa. This has resulted in a downward revision of Eswatini's forecasts, given its close ties with the South African economy. While there is not enough information currently about the risk of the coronavirus disease (COVID-19) in human immunodeficiency virus (HIV) populations, evidence from other viral respiratory infections suggest that COVID-19 may be very dangerous for populations with a large proportion of people with suppressed immunity such as from HIV and tuberculosis (TB) as well as those with high levels of poverty and malnutrition.

¹ The proportion of Emaswati living below the upper-bound poverty line of SZL 975.3 per adult equivalent per month (in January 2017 prices).

² 2018 Global Hunger Index.

³ EHIES (Eswatini Household Income and Expenditure Survey) 2017.

⁴ IMF (International Monetary Fund). 2019. *Eswatini: Exploring Options for Expenditure Rationalization. Technical Report.*

⁵ Kingdom of Eswatini Budget 2020/21 At a Glance, Budget and Economic Affairs Department, Ministry of Finance.



5. **On March 17, 2020, Eswatini declared a state of emergency. The primary crisis is one of health.** The fundamental concern is to contain COVID-19 and delay the spread of infection. The Government of Eswatini (GOE) recognizes that the cost of not acting could be detrimental and this includes actions beyond emergency response. Investments today in strengthening the health system are urgent and critical to protect Eswatini's economic and human capital potentials.

6. **Eswatini's National Development Strategy, Vision 2022, defines its aspiration to be in the "top 10 percent of the medium human development group of countries" and commitment to address issues of poverty and access to quality health care, gender equity, and social integration,** emphasizing technology use and innovation to help reach its vision.

7. **Recognizing the importance of human capital as a central contributor to sustainable economic growth and poverty reduction, in March 2019, Eswatini joined the group of early adopters of the Human Capital Project.** Despite its lower-middle-income status, Eswatini's Human Capital Index (HCI)—a composite measure of survival of under-five children, educational attainment, and adult survival rate and stunting—is low, on par with the Sub-Saharan average and lower-income countries (table 1). An HCI score of 0.41⁶ indicates that a child born today in Eswatini will only be 41 percent as productive when s/he grows up as s/he could be if s/he benefited from complete education and full health. Investing in human capital would increase the GDP per worker. To meet Eswatini's vision for human development, about a 15-percentage point increase is required from its current HCI score, calling for improvements in health, nutrition, and education services and cross-sectoral synergies.

8. **Human capital is also a priority for the World Bank.** The proposed project, in line with the GOE's strategic priorities, focuses on strengthening the health system and community platforms to help address critical human capital challenges, including stunting, child mortality, and adult mortality. The project also builds on prior support and lessons from Eswatini engagement and complements efforts by the GOE and the World Bank in other sectors to support the human capital agenda, including education and water supply and sanitation.

Table 1. Eswatini's HCI Performance, Benchmarked to Sub-Saharan Africa and Income Groups

Indicator	Eswatini	Sub-Saharan Africa	Low Income	Lower Middle Income	Upper Middle Income	High Income
	Male and Female					
HCI Component 1: Survival						
Probability of survival to age 5	0.946	0.934	0.929	0.961	0.983	0.995
HCI Component 2: School						
Expected years of school	8.2	8.1	7.8	10.4	11.7	13.3
Harmonized test scores	440	374	363	391	428	506
HCI Component 3: Health						
Survival rate from ages 15–60	0.588	0.732	0.745	0.807	0.863	0.923
Fraction of children under 5	0.745	0.684	0.658	0.730	0.869	0.935

⁶ The achievement on the HCI's health dimensions are probability of survival to age 5 = 95 percent, adult survival rate = 59 percent, and not stunted rate = 74 percent.



Indicator	Eswatini	Sub-Saharan Africa	Low Income	Lower Middle Income	Upper Middle Income	High Income
	Male and Female					
not stunted						
HCI	0.41	0.40	0.38	0.48	0.58	0.74

Source: World Bank, 2019, Human Capital Project.

B. Sectoral and Institutional Context

9. **Eswatini's HIV epidemic, persistently high maternal and child mortality, increasing noncommunicable diseases, and malnutrition affect its human capital formation.** Eswatini has the highest HIV prevalence in the world, with more than a quarter (27 percent) of its reproductive age population living with HIV. While the national HIV response has achieved significant success, particularly in its rapid scale up of antiretroviral treatment and evidenced by a 44 percent reduction in HIV incidence from 2011 to 2016, challenges remain. HIV incidence is the highest in the world and HIV remains the leading cause of death in the country. Furthermore, high levels of maternal and child mortality and stunting, coupled with a surge in NCDs, further drive Eswatini's performance on the HCI, particularly in relation to child survival (Component 1 of the Human Capital Index: Survival to age 5) and stunting and adult survival (Component 3 of the HCI: Health).

10. **The number of maternal and neonatal deaths is very high due to failure to provide high quality, responsive care to pregnant women and their babies in health facilities.** Despite increased access to antenatal care (ANC, 76 percent attend at least 4 visits) and women delivering in health facilities (88 percent), Eswatini's maternal mortality ratio remains very high (437 per 100,000).⁷ Most maternal deaths are linked to poor quality of care in facilities including lack of or poor monitoring, substandard management, and delayed intervention and referral.⁸ Quality of ANC is also a challenge.⁹ Almost all maternal deaths are deemed avoidable, with most deaths associated with obstetric hemorrhage (15 percent of deaths), hypertensive diseases of pregnancy (8 percent), and pregnancy-related sepsis (8 percent). While there has been better progress with neonatal mortality (17.2 deaths per 1,000 births), neonatal deaths still account for a third of the total under-five deaths. More than 600 perinatal deaths were registered in 2018, of which 27 percent were recorded as fresh stillborn, 38 percent as macerated stillborn, and 35 percent as early neonatal deaths, pointing to challenges across the continuum of care, from antenatal, delivery, and postnatal care. Beyond the neonatal period, mortality among under-five children is driven by diarrhea (20 percent of under-five deaths), lower respiratory infections (17 percent), HIV (11 percent), and protein-energy malnutrition (5 percent).¹⁰

11. **Adolescent girls and their babies are particularly vulnerable to poor outcomes.** High teenage pregnancy in Eswatini is a significant barrier to human capital accumulation. While the adolescent fertility

⁷ World Bank Open Data, 2018, accessed February 29, 2020.

⁸ Analytical and Advisory Services (ASA) Strengthening Capacity in Health Financing and Hospital Governance and Management in Eswatini (P163653); Eswatini MOH Confidential Enquiry on Maternal Deaths (2014-16).

⁹ Healthy Mothers, Healthy Babies: A Study to Assess Quality of Care in Maternity and Neonatal Services, supported by ASA P163653.

¹⁰ Global Burden of Disease 2017, Institute for Health Metrics & Evaluation, University of Washington.
<https://vizhub.healthdata.org/gbd-compare/>



rate has declined in recent decades, at 77 births per 1,000 girls (2017), it remains very high. It is more than double the rate observed in South Asia, for example, and up to 30 percent of all pregnancies in Eswatini are among adolescents. In the large-scale Sitakhela Likusasa Impact Evaluation (2016-2018),¹¹ almost half (44 percent) of adolescent girls and young women reported ever being pregnant. The study highlighted the importance of education on health and fertility; incentives paid to adolescent girls and young women to stay in school in Eswatini significantly reduced the odds of HIV acquisition by 23 percent (37 percent when combined with raffles) and significantly contributed to pregnant participants going back to school after they had their babies and to reductions in teenage pregnancy. The study also highlighted the need to strengthen family planning for adolescent girls and young women; more than half (56 percent) of the participants report having never spoken to anybody about contraception and one in five was not using any contraception despite being sexually active. In addition to its negative impact on girls' education (two-thirds of girls who drop out of secondary education in Eswatini do so because of pregnancy) and the consequent risk of HIV, teenage pregnancy significantly contributes to high anemia and malnutrition in adolescents, maternal and child morbidity and mortality, and to vicious intergenerational cycles of poor health and poverty. Adolescent girls who become pregnant are more likely to be socially isolated and suffer from depression. They are less likely to attend ANC (67 percent attend four visits versus 84 percent in women more than 35 years). Their infants are more likely to be born preterm, have a low birth weight, become stunted, and die as infants compared to infants born to older mothers. Infants born to adolescent mothers are also more likely to grow up in an unsupportive home environment, have poor cognitive development, drop out of school, be unemployed or underemployed, and if female, become pregnant in their adolescence, thereby cementing the perpetual intergenerational poverty cycle and contribute to the intergenerational cycle of malnutrition. Indeed, a strong negative correlation exists between fertility and human capital (-0.82) and GDP per capita (-0.78).

12. A triple burden of malnutrition (undernutrition, overnutrition, and micronutrient deficiency) adversely affects Eswatini's human capital.

- **A quarter of children under five years in Eswatini are stunted and 2 percent are wasted** (2014),¹² which is linked to poor breastfeeding practices and quality of complementary foods (low dietary diversity) especially among infants.¹³ Stunting is more prevalent in rural areas (27 percent) compared to urban areas (19 percent) and among children living in the poorest households (30 percent) compared to the richest households (9 percent).
- **The prevalence of overweight/obesity is high among women and children.** Nine percent of children under five years are overweight or obese, with prevalence being higher in urban areas compared to rural areas (12 percent versus 8 percent) and among children living in the richest (18 percent) compared to the poorest households (6 percent). Additionally, more than half of the women of reproductive age (51 percent) are either overweight or obese (18 percent among men).

¹¹ NERCHA and World Bank 2020. Conducted as part of the World Bank ASA HIV Incentives Evaluations in Swaziland and South Africa (P151327).

¹² Swaziland Multiple Indicator Cluster Survey 2014 (2016), Central Statistics Office.

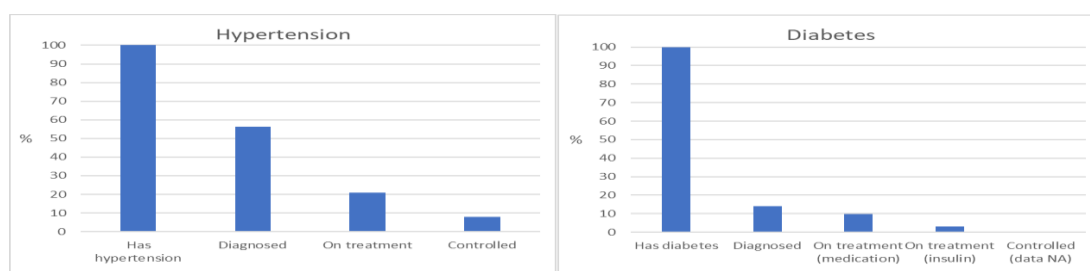
¹³ Multiple Indicator Cluster Survey 2014; SMART 2017.



- **The prevalence of micronutrient deficiency is also high among women and children.** The prevalence of anemia among women, pregnant women, and children under five years is 27 percent, 30 percent, and 42 percent, respectively.¹⁴
- **Malnutrition has a long-term negative impact on health, knowledge, skills, and productivity.** Malnutrition in the first years of life affects childhood survival, early childhood development, learning abilities, and educational outcomes in school, with long-term negative consequences for work productivity and economic development. In 2009, the estimated economic impact of child stunting—which was only marginally higher at the time (31 percent) than current estimates—was SZL 783 million (3.1 percent of GDP). Stunting places children at higher risk of NCDs in later life. Obesity also causes NCDs and leads to increased health care costs, reduced work productivity, increased disability, and premature deaths.¹⁵ Among women, it also contributes to premature deliveries, low birth weight, and other poor birth outcomes.

13. **The rapid increase in NCD prevalence has resulted in a high number of avoidable morbidities and premature deaths, negatively affecting human capital.**¹⁶ NCDs as a cause of mortality increased from 23 percent in 2007 to 40 percent in 2017, with most NCD-related deaths linked to cardiovascular diseases and diabetes and kidney diseases. Adult prevalence of hypertension and type-2 diabetes—key risk factors for cardiovascular diseases—is 25 percent and 14 percent, respectively, with significant drop-offs across the cascade of care (Figure 1), highlighting systemwide deficits, including failures in primary care and inadequate coordination and continuity of care.¹⁷ Currently, the majority of NCD service delivery, including uncomplicated cases, is provided by physician-led teams at tertiary-level facilities. Screening is done on an ad hoc basis. Furthermore, there are gender disparities in NCDs; while the prevalence of hypertension and diabetes is higher among women, men are less likely to seek care and achieve disease control.

Figure 1. Cascade of Care for Hypertension and Diabetes



Source: Author, using data from WHO STEPS 2014.¹⁸

Note: Hypertension prevalence = 25 percent; Diabetes prevalence = 14 percent; Population = 15–69 years.

¹⁴ Eswatini 2018 Nutrition Country Profile. Globalnutritionreport.org.

World Bank Open Data, 2016, accessed March 1, 2020.

¹⁵ World Bank. 2020. Obesity: Health and Economic Consequences of an Impending Global Challenge.

¹⁶ World Bank. 2019. Enhancing Human Capital and Boosting Productivity by Tackling Non-communicable Diseases: A Joint Agenda for Countries and Partners. The World Bank Group Human Capital Project.

¹⁷ Kruk et al. 2018. “High-quality Health Systems in the SDGs Era: Time for a Revolution.” Lancet Global Health. 6 (11): e1196–e1252.

¹⁸ World Health Organization (WHO), STEPS Noncommunicable Disease Risk Factor Survey is a household survey tool for collecting, analyzing and disseminating data on risk factors of non-communicable diseases (NCDs) and associated health system response at the population level.



14. The above health challenges are due to shortcomings in Eswatini's health system foundations that lead to poor quality care, as follows:

(a) Population health needs, expectations, and demand

- Eswatini's health system is not aligned with population health needs. In contrast with the focused efforts and demonstrated success of scaling up HIV treatment, health-specific nutrition and NCD care have been neglected.
- Client experience and satisfaction with health services is low. For example, a recent survey¹⁹ with mothers following their delivery at two hospitals—where more than a third of all institutional deliveries took place in 2018—found that 20 percent of the women interviewed reported that they would not return to the facility or recommend the hospital to family or friends despite being satisfied with their health condition and the health condition of their babies.
- Poor utilization of health services is linked to sociocultural beliefs and practices and lack of knowledge and awareness, which particularly affects early antenatal care and postnatal care attendance, malnutrition and NCD prevention and management, and adolescent health. To help address these challenges, community dialogues and targeted community-based and social and behavioral change communication (SBCC) interventions are required.

(b) Stewardship, governance, and financing

- Eswatini's health system performance is hampered by outdated legislation and weak regulation and accountability and management structures. The sector is overcentralized and there is limited capacity to institutionalize quality, particularly at the regional (Regional Health Management Teams [RHMTs]) and facility levels. Creating a culture of service-oriented teams across the central, regional, and facility levels to ensure quality of services and patient satisfaction requires strengthening the leadership of the Ministry of Health (MOH) to modernize the system, and strengthening institutions for evaluation, evidence-based learning, and improvement.
- While Eswatini's spending on health²⁰ increased between 2017 and 2019 (Figure 2A), macroeconomic challenges have led to reduced fiscal space for health. The FY20/21 health sector budget is SZL 2.2 billion (US\$ 126 million), up from SZL 2.0 billion (US\$ 140 million) from the previous fiscal year. This represents 10.5 percent of the total government budget. In real terms, however, the sector's budget has declined (Figure 2A). The breakdown by economic classification shows that personnel and drug expenses take up nearly two-thirds of the budget. Recurrent expenditures account for around 90 percent of the budget while capital expenditure and budget for maintenance are limited (Figure 2B). Administrative data suggest that spending on preventive and

¹⁹ Healthy Mothers, Healthy Babies: A Study to Assess Quality of Care in Maternity and Neonatal Services, supported by ASA P163653 (2018-2020).

²⁰ Per capita health expenditure was estimated at about US\$233 in 2016, which is above the income group average of about US\$60. Other estimates show around US\$248 per capita per year.



curative medicine is limited, respectively about 7 percent and 0.1 percent of recurrent expenditures. Furthermore, important areas such as nutrition and NCDs have been underfunded. Support from development partners (DPs) (approximately 24 percent of total health expenditures)²¹ is also expected to decline. While there is a Health Financing Policy,²² there is no clear health financing strategy and limited capacity to implement sustainable health financing mechanisms such as strategic purchasing that are critical for Eswatini to progress toward universal health coverage (UHC) and fulfill its human capital potential.

Figure 2A. Nominal and Real Health Expenditure Trend (FY14/15–FY18/19) (billions)

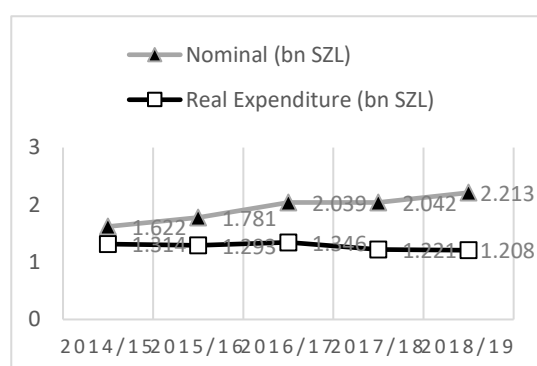
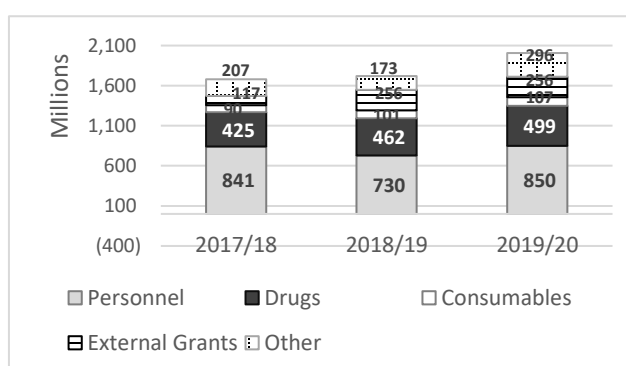


Figure 2B. Health Sector Budget FY17/18–FY19/20 by Economic Classification (millions)



Source: UNICEF Health Budget Brief 2018/2019 – Kingdom of Eswatini; GOE Budget Estimates 2020.

(c) Health system platforms and organization of care

- **The success lies in fighting Eswatini's HIV fragmented health system, resulting in service delivery silos with significantly different capacity, transparency, and performance.** Attaining the GOE's aspiration of a 'One Health System' requires investments to strengthen delivery platforms and the referral and supervision links between them.
- **Eswatini relies on several platforms for health service delivery**, organized in five levels (Figure 2.1). Primary health care (PHC) is provided by CHVs (Level 1) and 304 PHC clinics (Level 2). Secondary care is provided by five health centers (Level 3) and six hospitals (Level 4). Tertiary care is provided by three referral hospitals (TB Hospital, Psychiatric Hospital and Mbabane Government Hospital [MGH], Level 5).
- **Eswatini's community-based health services require strengthening.** Eswatini initiated community-based health services through rural health motivators (RHMs) in 1976 to address key health challenges that were most prevalent at that time. Eswatini has more than 5,200 RHMs and other CHVs—including those that work in urban areas and who

²¹ 2017/18 unpublished National Health Accounts, 2019.

²² The National Health Financing Policy was formulated to cover 2015–2022 (2015; updated in 2018).



are collectively known as CHVs²³—that form an integral liaison between health care facilities and the community. They are accepted and respected within their communities. However, lack of standardization in the training of CHVs and their responsibilities has resulted in fragmentation with different messages and services being provided to communities often causing confusion among communities. Due to this, as well the growing need for CHVs to provide more comprehensive PHC services, the MOH developed a standardized comprehensive training package for CHVs in 2018, to equip CHVs with the required knowledge, skills, and attitudes to provide community members the information and skills they need to access health care and ensure good health for themselves, their families, and their communities.

- **PHC facilities have been neglected.** In addition to improving the quality of care of the existing reproductive, maternal, newborn, child, and adolescent health (RMNCAH) services in PHC facilities, there is a need to integrate and scale up nutrition and NCD services—supported by strong referral systems and links to higher-level care—to significantly reduce avoidable morbidity and mortality in a more efficient manner. The MOH is committed to scaling up PHC-based NCD services nationally, based on a successful feasibility pilot conducted in 20 PHC clinics (10 control) in Lubombo region applying the World Health Organization Package of Essential NCD Interventions (WHO-PEN) for Primary Health Care in Low-Resource Settings. The aim is to scale up to all PHC clinics in the next five years, in a phased approach. However, this will require significant resources and support to the MOH, including from DPs.
- **The effective use of hospitals and quality of care in hospitals also requires attention.** Hospitals have limited service profile and quality of health services is generally low.²⁴ While several interventions have focused on unit/program-specific solutions, addressing the observed low performance at hospitals also requires strengthening hospital governance and management, and improving how hospitals are paid.²⁴

(d) **The availability of competent and motivated health workers**

- **Eswatini has a shortage of specialists and an outdated skill mix.** The MOH Human Resources for Health (HRH) unit has inadequate capacity and information for policy and strategy development and workforce planning, relying on outdated policies, strategies, and an Establishment Register.²⁵ Workforce planning is not customized to the changing epidemiological and service delivery context and there is inadequate production and retention of specialist doctors, nurses, dieticians, and public health nutritionists. While the country has an equipped neonatal unit, financed under a previous World Bank-

²³ Community health volunteers are paid a small stipend/incentive for their work

²⁴ Reports on Hospital Governance and Management (MGH, RFMH, Piggs Peak); Hospital Reform in Eswatini – A Summary Proposal; Hospital Strategy Assessment and Recommendations – RFMH; Micro and Meso-level Analysis of Hospital Governance and Management; Macro-level Analysis of HGM in Eswatini; and workshops supported under ASA P163653 (2018-2020). Healthy Mothers, Healthy Babies: A Study to Assess Quality of Care in Maternity and Neonatal Services, supported by ASA P163653.

²⁵ The Establishment Register is a document published by the Ministry of Public Services (MOPS) that defines the number and type of positions that the MOH can have.



financed health project²⁶ and by the United Nations Children's Fund (UNICEF), no neonatologists or neonatal specialist nurses have been trained to run the unit.

- **Competency, performance, and motivation among available health workers is a challenge.** The MOH has no standardized and objective framework for assessing and monitoring skills and performance. Lack of supervision is a ubiquitous challenge. There is also a challenge with in-service training and availability of job aids. Of health facilities providing ANC, for example, 63 percent do not have any staff that have been trained on ANC and only 59 percent have checklists or job aids (paper based) to facilitate high-quality service delivery.²⁷ A recent analysis conducted to assess the proportion of facilities that meet quality standards for adolescent friendly services found that only 35 percent of health care workers reported being trained on adolescent sexual and reproductive health services and only 8.5 percent recalled being trained on confidentiality and audio-visual privacy.²⁸ Others reported that they had forgotten the topics on which they were trained, given that they were trained a long time ago. With the new MOH guidelines on the provision of adolescent friendly services, the forthcoming new ANC model—based on eight rather than four visits—and the move to integrate and scale up nutrition and NCD management in primary care, training, supported by ongoing mentorship, supervision, and digital job aids to consolidate the learning and facilitate long-term adherence to competent care, will be even more critical.

(e) **Essential medicines, commodities, and equipment**

- **Stockouts of essential medicines and consumables is a challenge in Eswatini.** Paradoxically, facilities with oversupply of medicines and commodities that expire is also common. This is largely due to poor forecasting and supply chain management. To address this, Eswatini has developed an electronic Logistical Management Information System (eLMIS) that provides end-to-end visibility of stocks and movement of goods through the supply chain from the Central Medical Store (CMS) to facilities. This optimizes quantification, ordering, buffer stocks, and availability of medicines at the point of dispensing to the patient. However, this is currently only available at three facilities.²⁹ Furthermore, there is opportunity to improve efficiency by integrating supply chains (drugs, vaccines, laboratory commodities, and so on). For example, deliveries to facilities for drugs and laboratory commodities are currently planned and executed independently. While inefficient, this has been partly unavoidable due to small trucks that are unable to deliver everything required by the facilities in one delivery.
- **The delivery of essential services is also hampered by lack of functional equipment** due to poor tracking of available equipment and their maintenance and repair needs.

²⁶ Swaziland Health, HIV/AIDS, and TB Project (P110156).

²⁷ SARA (Service Availability and Readiness Assessments) 2017.

²⁸ UNICEF. 2017. *Eswatini National Adolescent and Youth-friendly Health Services*. Standards Implementation Assessment. Baseline Report.

²⁹ Piggs Peak Government Hospital, Dvokolwako Health Center, and Lobamba Clinic, with support from the Global Fund.



A Review of the Maintenance of Health Facilities and Equipment (2017)³⁰ and a National Equipment Inventory Report³¹ (2018) highlight challenges in tracking of equipment and servicing needs at facilities. This is due to lack of a live facility-based asset inventory system, manual recording of equipment inventory that are not kept at facility level, and manual recording of service requests from facilities with no system to minimize equipment downtime and trace service completion. This means that facility staff are not aware of the equipment they have and delayed response to maintenance and servicing needs can disrupt essential service delivery.

(f) Data and digital health technology

- **Eswatini’s electronic medical record system, known as the Client Management Information System (CMIS), is available in less than half of facilities (48 percent) and does not cover, or inadequately covers, all health conditions.** In addition to scale up of the CMIS, technical assistance (TA) in capacity building in data analytics and science to enable the most efficient utilization of the available data and in the maintenance of hardware, software, and data warehousing, is required.
- **In addition to the CMIS, other digital health interventions³² are not used extensively in Eswatini,** for example, for client communication, clinical case management, and health care provider decision support and referral coordination.

15. **To meet the policy objective of the National Health Sector Strategic Plan (NHSSP II 2019–23)—“build an efficient, equitable, client-centered health system for accelerated attainment of the highest standard of health for all people in Eswatini”—the MOH recognizes the need to invest in health system strengthening** to improve the coverage of essential services and the quality of care. The recent COVID-19 outbreak has underscored the importance of a responsive and resilient health system. Modernizing the foundations of the health system and applying evidence-based improvements are key to achieving better quality care and, through that, reduced child and adult mortality and stunting for human capital formation.

C. Relevance to Higher Level Objectives

16. **The proposed operation comes at a critical time, when the GOE has committed to a ‘turnaround strategy’ to attain macrofiscal stabilization and growth. One focus area of Eswatini’s Strategic Road Map (2019–23) is to improve the delivery of services and foster a culture of excellence** (improved efficiency and effectiveness of the public sector and technological innovation). The MOH has identified the priority areas for the project based on their contribution to Eswatini’s development objectives (Vision 2022; Eswatini Strategic Road Map, 2019–23).

17. **The proposed operation will support the MOH and the GOE to (a) progress with the implementation of NHSSP II (2019–23), which promises all Emaswati UHC, (b) improve service delivery, and (c) advance Eswatini’s human capital agenda,** including through the health and nutrition components

³⁰ Feasibility Study to Identify Options for Maintenance of Health Facilities and Equipment.

³¹ National Equipment Inventory Report. Biomedical Engineering Department, Ministry of Health, May 2019.

³² WHO (World Health Organization). 2018. *Classification of Digital Health Interventions v1.0. A Shared Language to Describe the Uses of Digital Technology for Health.*



of the HCI and cross-sectoral engagement. The project will complement and support significant government investments in these areas.

18. **The proposed project is complementary to the Eswatini COVID-19 Emergency Response Project (COVID-19 ERP P173883) and is designed to deepen and extend its investment impact through focusing on medium-term structural changes and the modernization of the health sector.** While the emergency operation focuses on short-term needs to support capacity in prevention, detection, and response to the threat posed by COVID-19, this operation offers a medium-term horizon to strengthen the health system's foundations and its preparedness to respond to the population's health needs, including through strengthened health facilities that can provide high-quality care to COVID-19 patients and those with chronic diseases who are particularly susceptible to severe COVID-19, digital platforms for real-time responsive data tracking and use, and governance and stewardship to efficiently and effectively manage the health response in such a dynamic environment.

19. **The proposed project is fully aligned with the adjusted Country Partnership Strategy (CPS) FY2015–2018, extended by two years (Report No. 892010-SZ);³³ the draft Systematic Country Diagnostic;³⁴ the Country Policy and Institutional Assessment (CPIA 2020); the Human Capital Agenda, and the IMF's assessment.** The proposed project will contribute to Pillar II (Strengthening State Capabilities) of the CPS, primarily through Objective 2 (Improved Social Services Delivery). It will coordinate with the other lending operations (Energy, Water and Sanitation, and Education)³⁵ to harness cross-sectoral synergies. Further, it connects to the two new subregional ASA tasks, which focus on the future of the medical workforce in Southern Africa (P171798; 2019-2021) and adolescent human capital challenges (P172420; 2019-2021). In response to the GOE's request to become an early adopter of the Human Capital Project, as an umbrella to the lending operations and ASAs, the World Bank prepared a cross-sectoral framework document³⁶ to guide Eswatini's approach to investing in human capital. This project addresses the health and nutrition aspects of the framework in coordination with the other sectors supported by the World Bank (Figure 2.2). Finally, the project is consistent with the IMF's Technical Report on Expenditure Rationalization (February 2019)³⁷ and the World Bank-supported Integrated Financial Management Information System and PFM Capacity Building Project (P159628).

20. **The project is aligned with the World Bank Group's twin goals of poverty reduction and sustainable shared prosperity and the World Bank's Global Strategy for Health, Nutrition and Population,³⁸** which supports countries' efforts to achieve UHC and provide quality, affordable health services to everyone, regardless of their ability to pay. Alignment with the twin goals and the sector strategy is supported by improving the coverage and quality of health and nutrition services for Eswatini's

³³ World Bank. 2018. *The Performance and Learning Review* (PLR – Report No. 126205-SZ); extended the CPS by two years to FY2020.

³⁴ World Bank. 2019. *The Kingdom of Eswatini. Toward Equal Opportunity: Accelerating Inclusion and Poverty Reduction in Eswatini*.

³⁵ Network Reinforcement and Access Project (P166170), Water Supply and Sanitation Access Project (P166697), and Basic Education and Skills Project (P173151)

³⁶ World Bank. 2019. *Investing in Human Capital in Eswatini: A Framework for an Integrated Multi-sectoral Support*. Draft, February, 2020.

³⁷ IMF. 2019. *Eswatini: Exploring Options for Expenditure Rationalization*. Technical Report.

³⁸ World Bank. 2020. *Refresh of World Bank Group Strategy for Health, Nutrition, and Population*. Draft Concept Note.



population, which contribute to UHC. In the long-run, investments under the proposed project will improve health outcomes, productivity, and lifetime and intergenerational effects on wealth and poverty.

II. PROJECT DESCRIPTION

A. Project Development Objective

PDO Statement

21. The Project Development Objective (PDO) is to improve the coverage and quality of key reproductive, maternal, neonatal, child and adolescent health (RMNCAH),³⁹ nutrition and NCD services (hypertension and diabetes) in Eswatini.

PDO Level Indicators

22. The PDO level indicators will be as follows:

- (a) Percentage of patients diagnosed with hypertension and diabetes who are managed at the PHC level (coverage)
- (b) Percentage of children under 5 years of age provided with a basic package of nutrition specific services (coverage)
- (c) Percentage of adolescents served or reached with quality sexual and reproductive health services (coverage)
- (d) Percentage of pregnant women receiving at least 4 ANC visits that meet defined quality standards (quality)
- (e) Percentage of health facilities that meet a minimum standard of quality as measured by a Health Facility Quality Index (quality)⁴⁰

B. Project Components

23. To address key sector challenges and support the achievement of the PDO, the proposed project would be financed by an IBRD loan of US\$20 million, using an Investment Project Financing (IPF) instrument, over a five-year period. The project will focus on strengthening the health system and ramping up investments in RMNCAH services as well as nutrition and NCDs (hypertension and diabetes) to address critical human capital challenges, including stunting and child and adult mortality, applying a life course approach. The project includes three components.

Component 1.	Improve health service delivery to increase the coverage and quality of health services to build human capital	US\$14.5
<i>Component 1.1</i>	<i>Build capacity of healthcare workers to deliver enhanced, high impact RMNCAH and nutrition and NCD services across the continuum of care</i>	<i>US\$3.0</i>

³⁹ RMNCAH is a package of services that is managed by the MOH Sexual Reproductive Health Unit.

⁴⁰ The index will be tailored to level of care to reflect differences in service profile at the primary and hospital levels.



<i>Component 1.2</i>	<i>Increase the availability of drugs, commodities, functioning equipment and client data for high-quality health and nutrition service delivery in facilities</i>	<i>US\$9.5</i>
<i>Component 1.3.</i>	<i>Strengthen the capacity of facilities, programs and regions to monitor quality of care, supervise and implement a Quality Management Approach in health facilities</i>	<i>US\$2.0</i>
Component 2.	Increase community demand for RMNCAH, nutrition and NCD services	US\$2.0
Component 3.	Strengthen the MOH's stewardship capacity to manage essential health and nutrition services and project activities	US\$3.5
Component 4:	Contingent Emergency Response Component	US\$0.0

Component 1. Improve health service delivery to increase the coverage and quality of health services to build human capital (US\$14.5 million)

24. This component will improve health service delivery to integrate and scale up quality nutrition and NCD services in PHC and improve the quality of RMNCAH services across the continuum of care. Under this component, the project will (a) build the capacity of health care workers to deliver high-quality RMNCAH, nutrition, and NCD services in PHC and higher-level care for continuity; (b) increase the availability of drugs, commodities, functioning equipment, and client data to support the delivery of these services through supply chain strengthening and investments in a digital health system; and (c) strengthen the capacity of facilities, programs and regions to monitor quality of care, provide supportive supervision and implement a Quality Management Approach (QMA) in facilities to ensure that the above inputs are translated into effective and high-quality service delivery.

Subcomponent 1.1. Build capacity of health care workers to deliver enhanced, high impact RMNCAH, and nutrition and NCD services across the continuum of care (US\$3.0 million)

25. This subcomponent will build the capacity of health care workers to deliver essential and high-quality RMNCAH, nutrition, and NCD services across the continuum of care through trainings, the provision of digital decision support tools ('digital job aids'), and the creation of a Community of Practice (CoP) for midwives as a platform for sharing best practices and creating new knowledge for continuous professional development.

26. Specifically, the following activities will be supported:

- **Competency-based and residential trainings for health care workers to deliver high-quality RMNCAH, nutrition, and NCD services across the continuum of care.** For PHC professionals, this will include training on the delivery of high-quality reproductive health services (family planning), antenatal and postnatal care, integrated management of childhood illnesses, and nutrition. To scale up NCD services in PHC, the training will be based on WHO-PEN for primary care in low-resource settings and will focus on NCD prevention, screening, diagnosis, and management, including stepwise treatment initiation and referral, and continuity in care. In secondary and tertiary level, training for health professionals including dieticians will include targeted competency-based training for midwives in comprehensive emergency obstetric and newborn care and in the skills necessary to implement the Baby-Friendly



Hospital Initiative⁴¹ to improve breastfeeding, as well as training on providing respectful maternity care and adolescent friendly maternity services. Doctors will also be trained on the new NCD PHC-based service delivery model including down referral to primary care. The dietetics training for dieticians (or nurses where dieticians are not available) would include dietary management of NCDs; dietary management of patients during critical care, pregnancy and lactation, and acute and chronic illnesses; infant and young child feeding including lactation management; anemia management among pregnant women and children; prevention of low birth weight among pregnant women; and management of severely and moderately malnourished children. For all trainings, focus on adolescent friendly strategies will be included. The training modules will also be developed on climate and health, setting out how climate-related exposures will affect vulnerable groups and the measures which need to be taken by health professionals to enhance resilience and adapt to these changes as well as mitigation measures that should be followed to reduce overall greenhouse gas (GHG) emissions from the health sector.

- **TA and procurement of tablets/technology to develop, test, and scale up provider digital decision support tools ('digital job aids') for RMNCAH to consolidate the trainings and ensure that competencies of health care workers are maintained for enhanced decision making.** These tools will help ensure that women, children, and adolescents receive high-quality safe care by facilitating health care worker adherence to guidelines through, for example, alerts and suggestions for next steps for investigation and treatments, improving risk assessments, adherence to service delivery packages, monitoring and timely and appropriate management and referrals. The tools would help facilitate supervision and accountability and will be linked with the CMIS.
- **Specialist training and mentorship to operationalize a four-bed specialized neonatal unit** in MGH, built and equipped through the previous World Bank-supported project. The project will finance specialist training in advanced midwifery and neonatology for six nurses as well as six-month fellowships for two pediatricians. Four international specialist neonatal nurses for three years will be recruited to provide ongoing training and mentorship to help the newly trained nurses apply their learning in clinical practice, increase their motivation, and cascade their skills and knowledge to other nurses to help mitigate against staff turnover. The objective is to operationalize the unit and slowly increase the capacity and level of specialization and bring the unit to a center of excellence in neonatal intensive care in the country.
- **Creation of a CoP for midwives to promote continuous professional development, intra-professional collaboration, and staff morale for improved maternal and neonatal outcomes.** This will cover financing for workshops to collectively define purpose, goals, and a vision for the CoP and operational costs, including meetings.

⁴¹ Launched in 1991, the Baby-Friendly Hospital Initiative is an effort by UNICEF and the WHO to ensure that all maternities, whether free-standing or in a hospital, become centers of breastfeeding support.



Subcomponent 1.2. Increase the availability of drugs, commodities, functioning equipment, and client data for high-quality health and nutrition service delivery in facilities (US\$9.5 million)

27. This subcomponent will increase the availability of drugs, commodities, functioning equipment, and client data in facilities critical for RMNCAH, nutrition, and NCD service delivery through strengthening of supply chain management and investing in a digital health system.

28. Specifically, the following activities will be supported:

- **Modernization and scale-up of an interoperable electronic record management system for service inputs (medical supplies, commodities, and equipment)⁴⁷ and service delivery and coverage (client records for clinical management) to strengthen evidence-based service planning and management.** The support covers the following tools and associated strengthening of analytical data science capacity to support their effectiveness:
 - The CMIS will be rolled out to 50 ‘last-mile’ PHC facilities to provide real-time data on RMNCAH, nutrition, and NCD services as well as TA to integrate a surveillance module for maternal, perinatal, and neonatal deaths as well as an inpatient module for hospitals. This surveillance capacity, focused on climate-vulnerable populations, will improve resilience by enhancing the ability of the system to detect and respond to new and re-emerging climate-related health outcomes and diseases.
 - eLMIS will be rolled out to 5 hospitals, 5 secondary health centers, and 10 prioritized high-volume sites at the primary level allowing timely ordering of essential medicines, drugs, and nutrition supplies in these facilities, and optimized quantification, ordering, buffer stocks, and availability of medicines at point of dispensing to the patient.
 - The Electronic Maintenance Management System (EMMS) and associated facility-based digital asset inventory will be developed and deployed allowing timely tracking of equipment and servicing needs of facilities to improve the availability of functional equipment required to provide quality health care.
- **Construction of an Integrated Operations Centre with upgraded fleet.** The project will construct and purchase related office equipment for an integrated Operations Centre to house the medical supply chain unit (managed by the CMS), a medical equipment maintenance workshop (managed by Biomed), and a data warehouse for information technology (IT) support (managed by the Health Management Information System unit [HMIS]). Given that the entities are servicing the same health facilities, this Integrated Operations Centre will allow effective and efficient support to health facilities by timely response to equipment downtime notifications, drug ordering requests, and hardware repair or IT support requests submitted by facilities. The design will be prepared in a way to contribute to climate co-benefits, including the use of energy efficient and sustainable technologies (for example, solar panel, insulation, refrigeration system, and LED lighting). The fleet will also be upgraded (including two larger trucks that have capacity to pool drugs, commodities, medical, and IT equipment; and cars to serve the four regions) to allow for



more efficient use of vehicles as the units will be able to coordinate routes through a joint fleet management system.

- **Limited procurement of key commodities and equipment for nutrition and NCDs** including blood pressure machines and cuffs, scales, height boards, micronutrient supplements (including iron and folic acid supplementation for adolescent girls), and therapeutic foods for children.
- **Procurement of biomedical equipment and supplies for hygiene and sanitation to supplement COVID-19 health care readiness in hospitals and health facilities**, including patient monitors, intensive care unit (ICU) beds, oxygen concentrators, waste skips, and disinfectant. These are complementary to procurements under the Eswatini COVID-19 Emergency Response Project to ensure medium-term system strengthening, the availability of essential inputs for sustained health care delivery, and mitigation of the impact of future epidemics.

29. **In terms of climate co-benefits, the activities under this subcomponent will make the system more efficient and resilient.** Digital health systems reduce the need for paper record keeping and are more responsive to changes in the epidemiology of climate-related health outcomes such as malaria, dengue, diarrhea, cholera, typhoid, and other vector-borne diseases. Improved fleet management and distribution means more efficient use of fuel and reduced carbon emission. Utilizing a professional maintenance model that reduces the need to replace equipment will reduce GHG emissions. The project will arrange for energy efficiency audits of existing health care facilities to be conducted. The recommendations from these will be implemented to improve the energy efficiency of buildings and equipment to ensure that future infrastructure and procurement practices are energy efficient. The audits themselves will be externally funded through an Energy Sector Management Assistance Program (ESMAP) grant with the World Bank energy global practice.

Subcomponent 1.3. Strengthen the capacity of facilities, programs, and regions to monitor quality of care, supervise, and implement a Quality Management Approach in health facilities (US\$2.0 million)

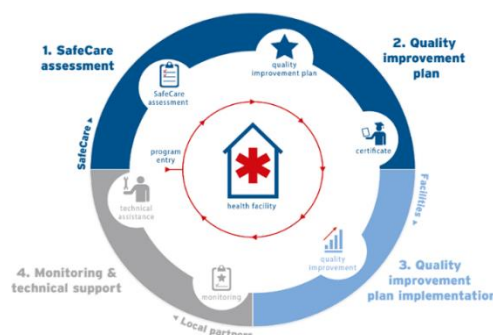
30. This subcomponent will help ensure that the above inputs translate into the delivery of high-quality services by strengthening the capacity of facilities, programs, and the RHMTs⁴² to supervise, monitor, and implement a QMA in health facilities. The QMA is central for outcome-oriented service organization and management, strengthening the link between health spending and health outcomes. Specifically, the project will finance TA and operational support to strengthen supportive supervision and mentorship for RMNCAH, nutrition, and NCDs, and implement a continuous quality of care improvement process⁴³ in facilities, externally validated through supervision by the RHMTs. Adolescent-friendly indicators—based on the country's Adolescent and Youth-Friendly Health Services' standards for health care facilities—will be included as quality indicators. Implementing the program will enable linking quality of care performance to an accreditation process.

⁴² The MOH's RHMTs are responsible for monitoring and supervising all health facilities and services within their respective region.

⁴³ Beyond the basic structural measures, this will include assessing processes, clinical outcomes, and reviewing the management and environmental health aspects of the facility to prepare tailored responses and actionable quality improvement plans.



Figure 3. Illustration of the Quality Improvement Program



Component 2. Increase community demand for RMNCAH, nutrition and NCD services (US\$2.0 million)

31. In addition to supply-side constraints, utilization of RMNCAH, nutrition, and NCD services is also affected by demand-side constraints, including insufficient knowledge on prevention and care seeking and cultural barriers. This component will strengthen the CHV Program, conduct targeted SBCC and develop client-based digital applications to address social and behavioral bottlenecks and generate demand for quality and service delivery uptake of RMNCAH, nutrition, and NCD services.

32. Specifically, the project will support the following:

- **Strengthening the CHVs Program to conduct community sensitization and outreach.** The project will finance the delivery of a comprehensive training package for CHVs and provide them with tablets in targeted areas to facilitate service delivery, monitoring and supervision. The training package will enhance community health volunteer skills in mobilizing communities, establishing support groups, organizing group counselling sessions, providing individual counselling for awareness generation and behavior change, identifying households through community listing and mapping to target sensitization and outreach based on household demographic profile and health risks, conducting home visits, and providing basic services. The training package will include gender-sensitive strategies to engage with communities as well as strategies to target other vulnerable groups, for instance, persons with disabilities. Adolescent-friendly strategies will also be employed. The training modules will incorporate climate and health, setting out how climate-related exposures will affect vulnerable groups and the measures that need to be taken by CHVs to enhance resilience and adapt to these changes as well as the mitigation measures that should be followed to reduce overall GHG emissions. As part of quality management, the project will also support the verification of community health volunteer performance by an inspector, a tested standard for other programs. Furthermore, for performance recognition, the project will support a national-level Community Health Volunteer Day.
- **Scale-up of social accountability monitoring of sexual and reproductive health (SRH) services.** This concept was developed by the Southern Africa Acquired Immune Deficiency Syndrome (AIDS) Dissemination Service in partnership with the MOH to transform the policy environment for positive Sexual, Reproductive Health and Rights outcomes among adolescents and young people (10–24 years) in Eswatini by 2023. The model supports



communities to generate data through a mobile application (Mobisite).⁴⁴ The data present community perceptions on the quality of public services being delivered and helps service providers identify service delivery gaps. The model was launched in 2018 and piloted in two sites (Lobamba and Manzini Clinics) with promising preliminary findings. The project will support scaling up the model to 20 new sites⁴⁵ across all four regions of the country.

- **Targeted SBCC.** The project will provide technical and financial assistance to design and deliver SBCC tailored to the target groups (pregnant women/mothers, adolescents, and those at risk for NCDs) relevant to the Eswatini context. The communication will focus on critical issues such as importance of family planning, early ANC seeking behavior, postnatal care, breastfeeding and complementary feeding, malnutrition prevention, child health, NCD risk factors, nutrition and lifestyle, screening, and adherence to management. Design of the SBCC will be based on formative research to discern health care-seeking behavior and its social and cultural dimensions, including those specific to adolescents as well as targeted communication to men to reduce the NCD gender gap. The SBCC will be implemented using various information platforms such as mass media (print, radio, video, social media, and internet) and group and individual counselling by the CHVs.
- **Design, develop, and scale up client-based digital applications to help generate awareness, improve knowledge, and boost uptake of services and adherence to appointments and treatment.** These will also be tailored to adolescents. Innovative and successful tools will be explored, such as ‘Mom Connect’ that has been successfully implemented in South Africa, to improve care seeking, health outcomes, and birth registration.⁴³ Digitalization and scale-up of the ‘Know your health numbers’ NCD campaign will also be supported.

Component 3. Strengthen the MOH’s stewardship capacity to manage essential health and nutrition services and project activities (US\$3.5 million)

33. This component focuses on strengthening the stewardship capacity of the MOH to manage health and nutrition services and project activities. It will also support engagement with the central agencies⁴⁶ and other line ministries that are critical for the implementation of the proposed sector strengthening and modernization to build human capital, supported by a human capital liaison. Specifically, the project will provide TA to support the drafting of updated regulations, policies, and strategies including for the draft Health Bill, professional regulatory bodies, National Quality of Care Framework, public-private partnerships, nutrition, sanitation, and health care waste management (HCWM)—including the capture or combustion of fugitive methane emissions—and health financing. TA and training/coaching for the MOH will also be provided on leadership and management to support modernization and organizational transformation of the MOH; strategic HRH planning; health planning and financing (including budget planning and monitoring and strategic purchasing); service contract development, negotiation, management, and revision; service delivery organization; climate change and health, and M&E. This will be complemented with twinning arrangements, particularly relevant during the first 12–18 months of

⁴⁴ www.mobisafaid.net.

⁴⁵ A site refers to an identified government health facility and its catchment of adolescents and young people who are receiving services there.

⁴⁶ Ministry of Finance (MOF), Ministry of Economic Planning and Development (MOEPD), MOPS, and MOH



project implementation. This component fosters investments in macro⁴⁷ and meso-level⁴⁸ strategies to generate lasting changes. The gradual reforms supported under this component are instrumental for the success of the interventions proposed under Components 1 and 2; it situates the micro-level⁴⁹ efforts as part of the broader reforms to sustain their impact.

34. **Project management and M&E. PROJECT**

Component 4. Contingent Emergency Response (CERC) (US\$0)

35. The project includes a CERC in accordance with the World Bank Policy: Investment Project Financing, paragraph 12 and 13, for situations of urgent need and assistance. This component will allow for rapid reallocation of project proceeds in the event of a future natural or manmade disaster or crisis that has caused or is likely to imminently cause a major adverse economic and/or social impact during the life of the project. Such events may include a disease outbreak. An assessment using the World Bank's Climate and Disaster Risk Screening Tool determined that Eswatini is vulnerable to risks because of climate change impacts including droughts, extreme temperature and inundation due to extreme precipitation and flooding. The CERC will have no funding allocation initially. In the event of a future emergency, this component would allow the GOE to request the World Bank to recategorize and reallocate financing from other project components to cover emergency response and recovery costs, if approved by the World Bank.

C. Project Beneficiaries

36. Direct beneficiaries are women of childbearing age, adolescent girls, newborns, and children under five years of age. Other direct beneficiaries will be populations at risk of NCDs and those living with NCDs. The indirect beneficiaries include the population of the four regions (Hhohho, Manzini, Lubombo, and Shiselweni). From an institutional perspective, health personnel, notably doctors, nurses, nutritionists, and midwives, and CHVs will benefit from training in RMNCAH, nutrition, and NCDs. The project will benefit management at all levels of the health sector, including the MOH, regions, and facility level. Various technical cadres will benefit from training to operationalize and sustain the new models and tools.

Table 2. Estimated Number of Target Beneficiaries over the Project's Timeline

Project Implementation Time	Baseline	Year 1	Year 2	Year 3	Year 4	Year 5	
Year	2020	2021	2022	2023	2024	2025	Estimated Total
Number of people receiving RMNCAH services	28,985	29,062	29,144	29,234	29,289	29,312	146,041
Number children receiving basic nutrition services	36,165	37,000	38,000	39,000	40,000	41,000	231,165

⁴⁷ Macro-level strategies are best able to directly tackle the social, political, economic, and organizational structures that shape a health system.

⁴⁸ Meso-level/subnational interventions address quality of care through the coordination and management of facility networks and communities.

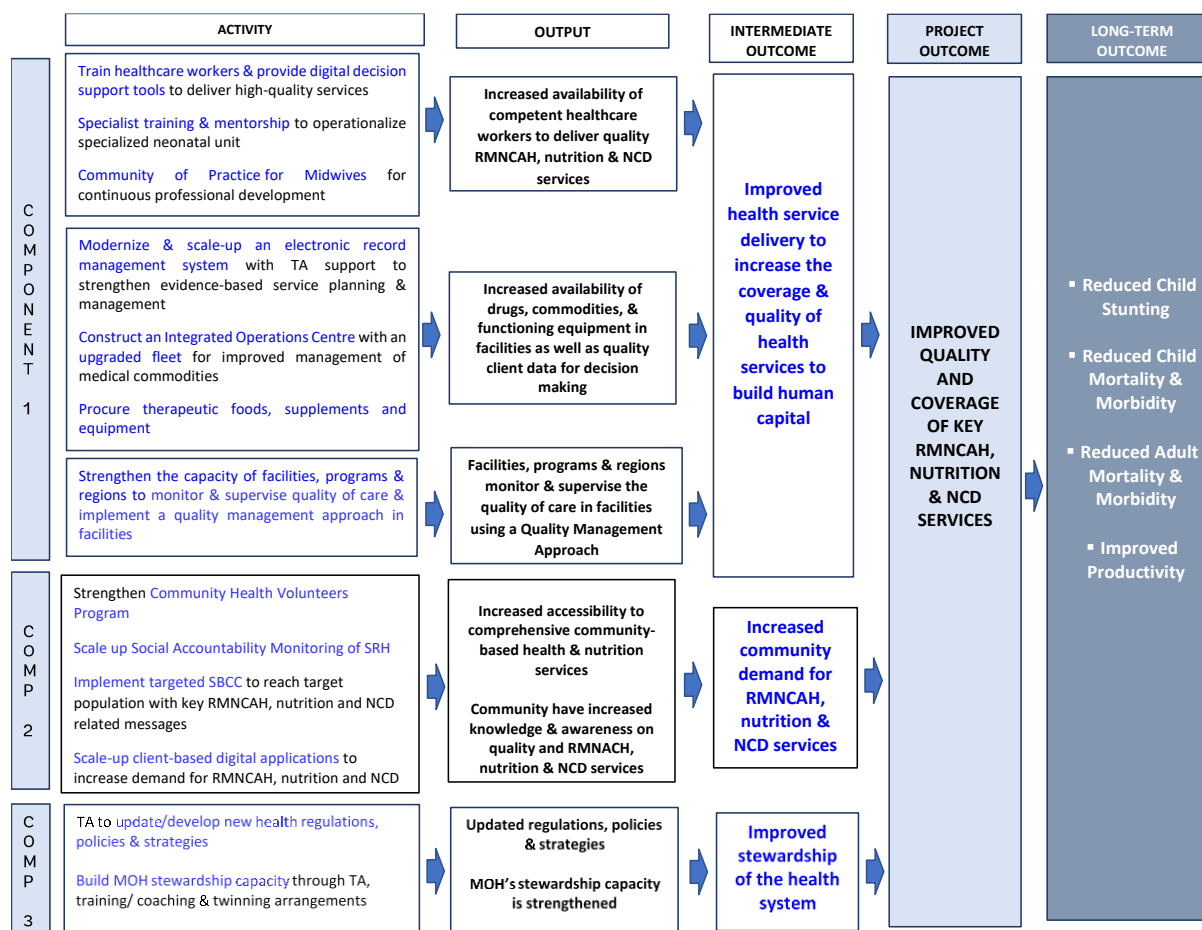
⁴⁹ Micro-level interventions focus on a narrow set of solutions, such as increasing health system inputs and changing people's behaviors and routines at the point of care, that is, at the lowest (micro) level of the health system (Kruk et al. 2018).



Number of people receiving NCD services	10,025	12,000	14,000	15,000	20,000	25,000	96,025
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D. Results Chain

Figure 4. Theory of Change



E. Rationale for Bank Involvement and Role of Partners

37. The project is proposed as a catalytic investment to enable the GOE to generate greater impact from its own resources, transforming the sector and facilitating sustained health system reform. The World Bank has the following comparative advantages: (a) experience in guiding complex reforms to strengthen sector stewardship and improve service delivery organization; (b) financing strategic capital investments, bundled with operational and technical support; (c) ability to guide and support a gradual move toward output-based financing (results orientation); (d) ability to engage across sectors, as well as with the private sector in health; and (e) experience in undertaking robust evaluation and capacity strengthening in applied data science to support decisions.

38. The fiscal space for health, in real terms, has been declining. In FY20-21, Eswatini is projected to spend 10.5 percent of its government budget on health. Support from partners, currently estimated



at around 24 percent of total health expenditures, is expected to decrease over time. The sector currently experiences repeated shortages of key inputs, pointing to inefficiencies in spending. In the face of fiscal pressures, unpredictable partner support, and uncertainties exacerbated by the COVID-19 epidemic, improving the sector's performance is critical. The World Bank can assist the GOE to manage this context, to improve sector performance and health outcomes for the available resources.

39. The proposed project design builds on a decade of engagement and lessons, including the Eswatini Health, HIV/AIDS and TB Project, which closed on September 30, 2018, and ongoing TA (P163653). As the first IBRD-financed operation in human development in Eswatini, the Eswatini Health, HIV/AIDS and TB Project played an important role in developing critical infrastructure to improve access to health care and strengthening the health and social protection systems between 2012 and 2018. To build on the momentum, in collaboration with DPs and sectors active in the World Bank's operational portfolio, the proposed project will be a strategic investment to catalyze sector modernization to improve service coverage and quality and serve the population better and more efficiently. The investment represents about 2.6 percent of the annual health sector budget, over a five-year period.

40. The World Bank has engaged DPs to map ongoing and planned technical and financial support and agreed on a coordinated approach to assist the MOH in its effort to implement the sector strategy and modernize the health system. The common thread of the consultative meetings is fourfold: (a) the system is currently fragmented and lacks a unified strategy for support from partners, which leads to 'patchwork-style' health system strengthening approaches, resulting in significant coordination costs and lack of alignment; (b) the system has shown successes and several programs or pilots have performed well; however, the know-how transfer from communicable disease management to NCDs and a 'full-system approach' is only slowly emerging; (c) the MOH and more broadly, the GOE is not well prepared for graduation from partner support, including financial and technical support. Considering the forecasted gradual reduction of partner support, the project can provide a platform to rethink sustainability and prepare for this challenge, complementary to the Global Fund to Fight AIDS, Tuberculosis, and Malaria's ongoing assessment of graduation readiness; and (d) the uptake from technical recommendations to implementation needs to be strengthened. DPs can jointly leverage action in agreed strategic areas to support the MOH to embark on implementation.

41. Bilateral support. Bilateral support is available from the US Government. The US President's Emergency Plan for AIDS Relief (PEPFAR) support to the national HIV and TB programs has been provided for the past 16 years. In 2020 this totaled US\$79.6 million, distributed through the US Department of State and implemented by the Center for Disease Control, Department of Defense, Peace Corps and the US Agency for International Development. Under the bilateral agreement, the United States plans to make available up to US\$225 million for the HIV response in Eswatini over the next five years. The funds will support strengthening Eswatini's multi-sector, health, social, and community systems, and build civil society and private sector capacity as implementing partners in the HIV response. The PEPFAR Country Operational Plan (May 2019) also focuses on the optimal configuration of investing in health systems, including supporting the development of a harmonized and interoperable HMIS. However, nationwide scale-up requires additional support, including capital investment. Lastly, PEPFAR support to HRH includes developing a uniform recruitment, remuneration, mentoring, supervision, and training structure. Additional support for operationalization is required. Taiwan, China through the International Cooperation and Development Fund (ICDF), its medical mission, and the Embassy, has been supporting the MOH by training medical personnel; implementing the Maternal and Infant Health Care Improvement



Project (now expanding into Phase 2); and infrastructure investment, including the Outpatient Department at the Mbabane National Referral Hospital, with an estimated total of nearly US\$13 million in 2019 and US\$6 million in 2018.

42. **Multilateral support.** Multilateral agency support includes the Global Fund to Fight AIDS, Tuberculosis, and Malaria and United Nations agencies. The Global Fund supports Eswatini with a US\$41 million grant (2018–2021), ‘Halting the spread of HIV and reducing TB prevalence in Eswatini.’ The Global Fund has been shifting increasing attention to the health reform agenda. A study is ongoing to assess the sector’s potential to reduce partner support and to inform improved system sustainability. The United Nations agencies (UNICEF, WHO, and United Nations Population Fund [UNFPA]) have been providing support in the areas of maternal, newborn, and child health, including family planning, nutrition, and prevention of mother to child transmission of HIV. UNICEF’s support to the MOH had budgets of US\$2.4 million (2018) and US\$2.2 million (2019). The WHO has been providing technical supporting for system strengthening (health financing, normative guidance for service delivery, and HRH). The WHO/UNFPA, together with ICDF, have been supporting strengthening basic emergency obstetric and neonatal care at lower-level facilities.

F. Lessons Learned and Reflected in the Project Design

43. The design of the project has considered lessons learned in the implementation of the Health, HIV/AIDS, and TB Project. The key lessons reflected in the design of this project are discussed in this section.

44. **Strengthen MOH ownership and capacity retention and improve sustainability.** During the implementation of the previous project, ownership and strengthening and retaining capacity within the MOH were challenged. This affected the institutional sustainability of outcomes. To reduce these risks, this project proposes (a) early engagement at senior policy making and political levels to build project ownership; (b) establishment of a PIU that is more closely integrated within the MOH; the design of the institutional arrangement is based on lessons from the closed operation⁵⁰ and assessments and consultations with the MOH; and (c) specific investments in the MOH, regional, and facility-level capacity strengthening, including technical and managerial skills and using approaches that ensure improved retention of skills (for example, twinning arrangement; capacity building is explicit in the Terms of Reference for TAs and is monitored, and so on).

45. **Shift the focus from access to quality, from structures to processes and patients.** Unwarranted focus on access and structures, without adequate emphasis on health service quality and patient outcomes, has led to unproductive or underutilized infrastructure or equipment. In response to this and moving along the development trajectory, this project proposes to focus on quality of care, including through strengthening service organization, quality management processes, and linking financing to quality.

46. **Secure critical inputs for effectiveness.** Under the previous project, due to structural problems, the absence of critical inputs, such as human resources and drugs, hampered effectiveness. The lesson

⁵⁰ Implementation Completion and Results Report: Swaziland Health, HIV/AIDS and TB Project (Report No.: ICR4723, March 2018)



was that critical inputs must be either directly procured/secured under the project or agreed with the MOH or DPs as a quasi-effectiveness condition. Hence, this project includes the production of some critical inputs (for example, specialists) and proposes broader structural measures (for example, strengthening HRH planning and management and improving stock and equipment security through integration and electronic management systems). Moreover, the project will monitor the availability of critical inputs and raise issues early on with the MOH and the central agencies.

47. **Invest in system foundations for sustainable impact.** The sustainability of outcomes has been hampered. This stems largely from unjustified focus on small-scale, fragmented approaches and inadequate investments in the foundations of the health system. This project fosters investments in macro- and meso-level strategies to generate changes that last. To engender sustainable impact, the project will support institutions and capacity and bolster the use of strengthened regulatory mechanisms and economic instruments (for example, contracts). Lastly, as health systems are complex adaptive systems, to guide implementation, investments, and policy making, the project will strengthen the evaluation function.

48. **Strengthen coordination and collaboration with non-health sector ministries that are critical for effectiveness.** The project acknowledges the potential contribution that other non-health sector ministries can make toward the achievement of the project's development objectives, particularly in the areas of human capital. To ensure coordination and collaboration with the related ministries, the project's institutional arrangements include two interministerial bodies: (a) Interministerial Technical Committee and (b) Interministerial Policy Advisory Group. For these bodies to be operational, they are proposed to be run on currently well-functioning administrative and decision-making structures.

III. IMPLEMENTATION ARRANGEMENTS

A. Institutional and Implementation Arrangements

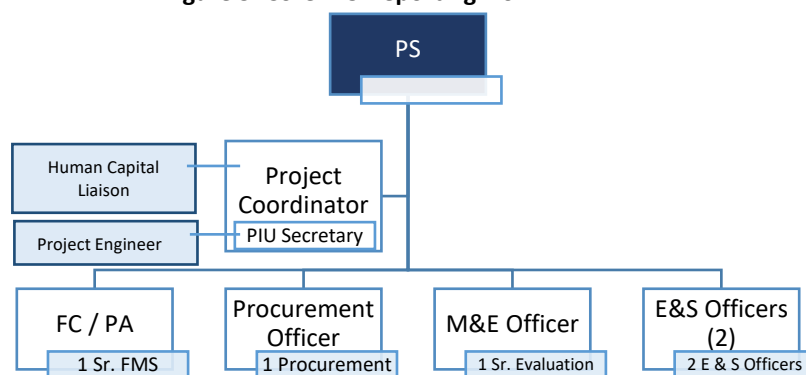
49. **Project management.** The project will be implemented by the MOH with the support of a PIU. To enhance coordination and ownership, the PIU will be housed within the MOH and integrated in the MOH's operational and management structure. The PIU will report to the Principal Secretary (PS). The primary functions of the PIU will be to coordinate effective implementation of the project and serve as the vehicle for capacity building and skills transfer to MOH staff in the areas of FM, procurement, environmental and social (E&S) risk management, and M&E. The PIU will consist of at least seven full-time MOH staff (project coordinator, financial controller, principal accountant, senior procurement officer, M&E officer, E&S officers, supported by a secretary) and seven technical staff to be hired under the project (that is, human capital liaison [1], senior FM specialist [1], procurement officer [1], senior evaluation officer [1], E&S officers [2], and oversight project engineer [1]). An overview is provided in Figure 5. The technical support on FM and M&E is for the duration of the project. The human capital liaison would support the MOH for the first 24 months of the operation; a critical time for cross-sectoral coordination and operational planning. The liaison will provide interface between the key human capital components, assuring cohesion with education, social protection and health. For procurement and E&S management, support is anticipated to be for the first 18–24 months of project implementation to accelerate implementation and provide sustainable knowledge transfer to MOH staff. To support the E&S development agendas in a sustainable manner, E&S will be integrated at the regional and community levels, using the existing



platforms and cadres (for example, regional health inspectors, CHVs, and social accountability officers). The PIU will be headed by a project coordinator, who will facilitate integrated implementation between project components and the activities of the MOH. The project coordinator will support harmonized policy, strategy, and operational processes that span departments/functional areas.

50. For operational efficiency, the PIU technical roles (fiduciary, M&E, and E&S) are shared between this project and the project implementation team for the Eswatini COVID-19 Emergency Response Project, which was approved on April 20, 2020, and became effective on April 30, 2020. In April 2020, the MOH assigned employees to establish this shared PIU. Sharing MOH staff and technical consultants will improve the strategic scope while reducing the overhead costs across the two projects. Tapping into technical surge capacity early on will accelerate effectiveness. The coordinators for the two projects (P173883 and P168564) will ensure that the project implementation responsibilities are organized so as to harness operational benefits and reduce operating costs. The two coordinators will present (monthly) on project progress to the Senior Management Team of the MOH.

Figure 5. Core PIU Reporting Flow



Note: FMS = Financial management specialist.

51. **MOH technical leadership.** To strengthen technical capacity in areas that are critical for project effectiveness and to enable and equip the MOH to manage sector modernization, the following departments/units will receive initial technical assistance (surge capacity) to support design and implementation: (a) HRH Unit: one senior technical assistant to support HRH strategy development and implementation and an HRH analyst; (b) Health Services Directorate: PHC focal team to catalyze primary care revitalization and improve service delivery organization; (c) Health Planning Unit: one senior technical assistant to support health financing/strategic purchasing; and (d) Strategic Information Department: one senior technical assistant to support data flow, data generation, and data analysis for decision making. The assistance will include skill transfer and support transition management. This technical assistant would also provide guidance for the M&E officer in the PIU to ensure robust project evaluation. An overview is provided in Figure 1.2.

52. Project governance beyond the MOH includes an Interministerial Technical Committee, to be convened and chaired by the Director of Health Services, MOH, and an Interministerial Policy Advisory Group that would consist of the PSs of the MOH and central agencies and report to the Secretary to the Cabinet to inform Cabinet and Prime Minister-level engagements. An overview is provided in Table 1.6.

- **Interministerial Technical Committee** will guide project implementation and its



operationalization at the technical level. To avoid creating parallel structures within government administration, this committee will consist of the sectoral officers responsible for health in the Ministries of Finance, Economic Planning, and Development and Public Service, together with their counterparts in the line ministries that are relevant for the project's success, including the Director of Health Services, MOH (Chair); the Director of Management Services Division, MOPS; the Director of the Ministry of Information Technology (MICT); and from ministries related to human capital development. The Interministerial Technical Committee will be convened and chaired by the Director of Health Services in the MOH, supported by the Project Coordinator and the human capital liaison, who will provide operational-level coordination between the PIU, the MOH, and other ministries/agencies relevant for human capital development. The liaison will provide a critical interface between the key human capital components, assuring cohesion with education, social protection, and health. The scope of this body will be to review and discuss technical aspects that require coordination with the central agencies and with line ministries and prepare joint reports/proposals for the Interministerial Advisory Group, for its information, guidance, or decision.

- **Interministerial Policy Advisory Group** will consist of the PS of the MOH (Chair), the central agencies, and the chief economist of the MOEPD. The role of this body will be to inform Cabinet-level decision-making and policy formulation, in alignment with Eswatini's development priorities. The Policy Advisory Group will report to the Secretary to the Cabinet to inform Cabinet and Prime Minister-level engagements. The project coordinator, assisted by PIU staff, will serve as the secretariat for the Policy Advisory Group.

B. Results Monitoring and Evaluation Arrangements

53. **The Results Framework will support the routine monitoring of project results and progress toward the PDO and intermediate results indicator targets.** The MOH will be responsible for ensuring the collection of data and reporting of all indicators regularly.

54. **The MOH, through the PIU, will monitor, verify, and report on the achievement of results, with the necessary protocols for each indicator, on time and comprehensively. A senior M&E adviser will be recruited to ensure effective monitoring of project activities and support the reporting function of the PIU (Component 3).** The M&E Plan, along with draft protocols to monitor achievement of quality standards, will be elaborated in the Project Operations Manual. The project will harness the HMIS (routine monitoring) to monitor and track project progress at the national, regional, facility, and community levels. The project will build upon existing client feedback mechanisms, managed by the MOH Quality Management Program, to report on beneficiary satisfaction rates as outlined in the Results Framework.

55. **The project aims to strengthen the GOE's M&E capacity, including the institutionalization of reporting related to service delivery, financial reporting, expenditure and disease analysis, and the adoption of an integrated HMIS** (CMIS, eLMIS, EMMS, and Human Resources Information System [HRIS]) that can be linked to service delivery and input planning, including health workforce planning. With the project's support, at least two rounds of quality and outcome surveys will be conducted to assess project effect on service delivery and quality of care. The project will support the design and implementation of the proposed activities through M&E, and operational research where applicable. In coordination with



other sectors/projects, evaluation is considered to gather evidence on the impact of the project together with interventions by other sectors/projects on human capital.

C. Sustainability

56. To meet the ambition of the NHSSP II (2018–23) under rising budget pressures, and to take the health system to the next level, the MOH has committed to undertake strategic service delivery reorganization and strengthening the link between funding and outcomes to improve care quality, better respond to clients, and use resources more efficiently. Shifting the focus from pure infrastructure investment to patient-oriented service delivery and broadening the scope from a fragmented, vertical approach to a system-wide strategy requires (a) strong political support at the highest levels, (b) institutional strengthening, and (c) effective partner coordination.

57. Investments in health services that aim to improve child and adult survival and stunting will not be effective and sustainable without investing in the health system's foundations. To enable a well-prepared and managed steady transformation process and to warrant effectiveness and sustainability, the project will support strengthening technical skills, managerial skills, and organizational changes, including change management.

58. Lessons from the previous World Bank-supported operation direct attention to the following areas to mitigate sustainability challenges: (a) competent HRH availability and strategic deployment; (b) coordinated planning and availability of all essential inputs required for service delivery (HRH, equipment, drugs, and commodities); (c) advanced planning for financial sustainability (transfer and absorption of recurrent costs); (d) assessments to inform reform design to commence early (during project preparation and the first year of project implementation) to allow for demonstrations, calibration, and evidence-based decisions for scale-up and (e) a PIU that is better integrated within the MOH and builds sustained capacity.

59. The project incorporates measures and activities to mitigate the institutional and financial sustainability risks (for example, institutional and organizational strengthening; use of existing platforms; budget-neutral design where possible, and an integrated PIU with an added layer of sustainable capacity strengthening).

IV. PROJECT APPRAISAL SUMMARY

A. Technical, Economic and Financial Analysis

60. **The economic analysis conducted for the project shows a strong economic rationale for the proposed investment.** The project is expected to improve coverage, quality, and systems for health and nutrition services for Eswatini's population through improving quality of health service delivery to build human capital; improve responsiveness of health services by investing in digital platforms and infrastructure; and strengthen governance and stewardship. The net economic benefits generated by the project's inputs and outputs resulted in a positive net present value (NPV) of between US\$22 million and US\$68 million, an internal rate of return of 9 percent, which was significantly higher than the current costs of capital in Eswatini (2.5–5 percent), and a cost-benefit ratio of 2.3 to 4.8, under the base-case scenarios



(2.5–5 percent discount rates) even with a conservative approach in estimating the benefits from the project.

61. **The analysis suggests that with an investment of US\$20 million over 5 years, around 3,900 mothers, children under five years, and neonates lives will be saved and potential productivity of 7,000 people will be preserved in Eswatini.** Mortality and morbidity impacts are estimated based on the number of pregnant women receiving at least four antenatal visits, children under five receiving a basic package of nutrition-specific services, and people who are screened and treated for NCDs. The cost-benefits analysis (CBA) is built directly on the PDO and its indicators, while recognizing the greater complexity of the underlying causal chain of the interventions on the outcomes and economic benefits. The project's effectiveness/benefits were estimated separately for each project component and then transformed into a monetary value in aggregated form.

62. **Sensitivity analyses show that the results of the CBA were sensitive to changes in key modelling assumptions, but the main conclusions remain unchanged, that is, the investment was justified on economic grounds.** Even under more conservative assumptions of variations of (a) economic growth, (b) coverage of interventions, and (c) discounting factors, the benefits still outweighed the costs. A higher discount rate of 7 percent reduced the Cost-Benefit Ratio (CBR) and NPV (CBR: 1.4, NPV: US\$7 million). To reflect uncertainty in economic growth, especially in a country with considerable political instability, the analysis was done assuming the lower bound of IMF projection, a long-term economic growth of 0.5 percent. Even under this unlikely scenario, the CBA showed that the investment yielded positive economic returns: a CBR of 4.2 per US dollar spent and an NPV of US\$68 million. The project assumed 80 percent coverage; however, the analysis included 50 percent coverage should there be implementation delays or failure to meet standards. With a lower coverage (50 percent), the conclusion remained unchanged.

63. **Beyond the CBA, at the system and program levels, the project will support improved efficiency within the sector (that is, doing the right things and doing things well) and therefore, better use of public resources.** These improvements are expected as a result of service delivery reorganization (doing things at the appropriate level of care) and improved planning, management, and productivity. For the consumer, benefits will accrue through the health (life expectancy and healthy years) and economic channels (productivity and earnings), and potentially over the life cycle (early childhood nutrition) and intergenerationally (maternal nutrition). These additional benefits are not accounted for in the economic evaluation but will be subject to assessment during the project's lifetime to inform decision making.

B. Fiduciary

(i) Financial Management

64. The proposed FM system and arrangements for the project will be capable of managing FM aspects for the project. The proposed PIU will be strengthened by the newly recruited FMS (proposed but not yet on board), seconded by the principal accountant from the MOH. The FMS will report to the project coordinator and the financial controller in the MOF for business continuity. The project will use the existing entire accounting system for recording and reporting on the project activities, and the internal control environment will be governed by the Public Financial Management Act (PFMA) of 2017, supplemented by the Project Operations Manual (POM).



65. The funds will be disbursed from the World Bank into a US dollar segregated Designated Account opened by the MOF at the Central Bank of Eswatini. A second Designated Account will be opened specifically for the CERC component as and when the component is activated. The project will use advance disbursement method based on six-months expenditure forecast. Other disbursement options such as reimbursements, direct payments, and special commitments will also be available if the need arises.

66. The project will produce quarterly unaudited interim financial reports (IFRs) to be submitted to the World Bank 45 days after the end of the financial quarter. The project's annual audit report and the auditors' Management Letter and management's response are to be submitted to the World Bank within six months of the end of each reporting period, that is, by September 30, each year. The FM arrangements meet the World Bank's minimum requirements under Bank Policy: Investment Project Financing. See annex 1 Financial Management for more details.

(ii) Procurement

67. All procurement to be financed under the project will be carried out in accordance with the World Bank Procurement Regulations for IPF Borrowers (dated July 2016, revised November 2017 and August 2018) and the provisions stipulated in the Legal and Project Agreements. Project procurement will be carried out by the MOH Procurement Unit. The unit will designate a person who will be a focal point for the project procurement. Project implementation will be carried out in accordance with the 'Guidelines on Preventing and Combating Fraud and Corruption in Projects Financed by IBRD Loans and IDA Credits and Grants', dated July 1, 2016 (Anticorruption Guidelines).

68. Procurement capacity and risk assessments were carried out to review the MOH Procurement Unit's organizational structure for implementing the project and the interaction between the project's staff responsible for procurement duties and management of the ministry. The assessment identified the following issues:

- (a) The MOH does not prepare an annual Procurement Plan, and the lack of proper procurement planning may lead to implementation delays.
- (b) During the assessment, the unit indicated that they communicate the outcome of the tenders to those that participated in the bidding process including reasons for failure. Unsuccessful bidders are also allowed to seek feedback on how they fared against the evaluation criteria if they so request.
- (c) The current procurement unit has an establishment of ten positions but only eight positions are filled, with two yet to be recruited. The staff have limited experience with World Bank procurement procedures.
- (d) The unit is currently housed at the CMSs and does not have basic operational equipment such as internet, office equipment, information and communication technology equipment, and storage space for filing.

69. Based on the preliminary assessment and taking note of the role and responsibility of the MOH for procurement, the procurement risk rating is assessed as 'Moderate'. The procurement unit requires



strengthening of its procurement and contract management capacities and the World Bank team will provide the needed support and training.

70. Risk mitigation measures include (a) hiring a qualified and experienced procurement professional to support and build capacity of the unit for 18 months; (b) the MOH filling the remaining two positions for the procurement unit; (c) monitoring the execution of the Procurement Plan through Systematic Tracking of Exchanges in Procurement (STEP); (d) debriefing of unsuccessful bidders upon request, consistent with the Procurement Regulations for IPF Borrowers; (e) putting in place a contract management plan for major contracts; (f) providing the unit with basic operational equipment, (g) training new and current staff on the World Bank Procurement Regulations, and (h) strengthening the unit by supporting the implementation of the actions summarized in Annex 1.

71. **A Project Procurement Strategy for Development (PPSD) has been developed to determine the approach to market, the selection methods, evaluation options, and sustainability considerations that may need to be included in the project's procurement processes.** In 2017, the GOE through the Office of the Prime Minister set up an interministerial task team to look at the state of service delivery in the health sector including procurement. The team identified some weaknesses and made recommendations and corrections, which were approved by the Cabinet. The study identified some key procurement performance issues (see Annex 1), including high and continually escalating prices for drugs; framework contracts not awarded to the lowest bidders for fear that they will renegotiate the prices once awarded the tender (incentivizes suppliers to quote higher prices); and significant delays in payments to suppliers, some for more than 365 days leading to (a) suppliers inflating prices to cushion the delays in payment and (b) few suppliers willing to do business with the GOE. In addition, there is weak contract management and poor management of maintenance of equipment. These weaknesses will be addressed under Component 3 of the project to ensure institutional capacity building and improvement in effectiveness and efficiency of health sector procurements.

72. The PPCSD will consider these and other factors in determining the Procurement Plan, especially the packaging given the small local supplier base.

73. The Procurement Unit has prepared a Procurement Plan acceptable to the World Bank. The plan will be uploaded into the STEP system, a planning and tracking system that will provide data on procurement activities, establish benchmarks, monitor delays, and measure procurement performance. The Procurement Plan will include (a) a brief description of the activities/contracts to be procured during the first 18 months of project implementation, (b) the approach to market and selection methods to be applied, (c) cost estimates, (d) time schedules, and the World Bank's review requirements. CERC procurement follows the World Bank's Guidance Note on Procurement in FCV and Situations of Urgent Need.

74. The Eswatini Public Procurement Act 2011 has been assessed and indicates that the country's regulations are generally consistent with international best practices, although some weaknesses were identified, which should be mitigated through adequate measures to ensure that (a) contract documents have an appropriate allocation of responsibilities, risks, and liabilities; (b) contract award is published; and (c) the national regulations do not preclude the World Bank from its rights to review procurement documentation and activities under the financing.



75. The Request for Bids/Request for Proposals document shall require that bidders/proposers submitting bids/proposals present a signed acceptance at the time of bidding, to be incorporated in any resulting contracts, confirming application of, and compliance with, the World Bank's Anticorruption Guidelines, including without limitation, the World Bank's right to sanction and the World Bank's inspection and audit rights.

76. With the incorporation of the abovementioned provisions, the Eswatini Public Procurement Act will be acceptable to be used under those procurements using the open national approach not subject to the World Bank's prior review as agreed with the World Bank in the approved Procurement Plan.

77. **Procurement of goods, works, and non-consulting services.** Goods and non-consulting services to be procured under this project will include equipment and electronic monitoring systems.

78. Procurement, while approaching the international market, will be done using the World Bank's standard procurement documents. Procurement while approaching the national market will be done using the national standard bidding documents, subject to incorporation of the abovementioned provisions, with an additional annex to address the World Bank's Anticorruption Guidelines and ensure universal eligibility.

79. **Procurement of consultancy services.** Consulting services to be procured under the project include hiring of firms to carry out studies, assessments, and related activities. Hiring of individual consultants will be limited to international consultants required for project implementation.

80. **Operating costs.** These items will be procured using the borrower's national procurement and administrative procedures acceptable to the World Bank, including selection of project implementation support personnel.

81. **Record keeping.** All records pertaining to award of bids, including bid notifications; register pertaining to sale and receipt of bids; bid opening minutes; bid evaluation reports; and all correspondence pertaining to bid evaluation, communication sent to/with the World Bank in the process, bid securities, and approval of invitation/evaluation of bids would be retained by the MOH and uploaded in STEP.

82. The following details shall also be published on the United Nations Development Business online and the World Bank's external website: (a) an invitation for bids for procurement of goods and works following open international market approaches, (b) request for expression of interest for selection of consulting services following open international market approaches, and (c) contract award details of all procurement of goods and works and selection of consultants using open international market approaches.

83. **Fiduciary oversight by the World Bank.** The World Bank shall prior review contracts according to the prior review thresholds set in the PPSD/Procurement Plan. All contracts not covered under prior review by the World Bank will be subject to post review during implementation support missions, including missions by consultants hired by the World Bank or through supreme audit institutions as part of the financial audit. The World Bank may, at any time, conduct independent procurement reviews of all the contracts financed under the credit.



84. **Contract management.** The MOH will identify high-value contracts for increased contract management support and indicate them in the Procurement Plan. The MOH will develop key performance indicators for such identified contracts and the indicators will be monitored during the actual execution of contracts. The World Bank team will provide additional due diligence and independent review of the contract performance of such identified procurements. A fully staffed project team of the MOH will be responsible for overall project/contract management.

	Triggered?
Projects on International Waterways OP 7.50	No
Projects in Disputed Areas OP 7.60	No

85. Operational Policy (OP) 7.50 is not triggered because the small-scale wastewater management/sanitation facilities will not have an impact on international waterways.

C. Environmental and Social

86. **The environmental and social risk classification of the project is Moderate under the World Bank's Environmental and Social Framework (ESF)** based on the type and nature of the project that will support strengthening of health systems for human capital development in Eswatini. Project activities and interventions involving support to improving sector governance and performance, health financing, service delivery, supply chain management, infrastructure and equipment maintenance, and health management information systems will require: (a) procurement of equipment, including electronic equipment and (b) capacity building and training in the application of analytical computing skills, setting up and use of various computer-based and other management systems, and the use and maintenance of infrastructure and equipment. The environmental impacts associated with these activities will result from the generation and disposal of electronic waste (e-waste) which is considered to have moderate environmental risk and general waste (paper, packaging, redundant non-electronic equipment, food waste, and so on) considered to have low risk.

87. **The Operations Centre will be built on the premises of the existing CMS site entailing minor construction works.** Support for hospital and health facility operations through the procurement and deployment of biomedical equipment and supplies for hygiene and sanitation is anticipated to result in risks and impacts related to the management and disposal of health care waste, occupational health and safety (OHS) of health care workers, and community health and safety related to the operation of health facilities.

88. **The operational support to hospitals and healthcare facilities through the procurement and deployment of this biomedical equipment will be implemented over a medium-term horizon to strengthen the health system.** So, while it may supplement COVID-19 healthcare readiness over the medium term, it will not be exclusive to COVID-19 treatment and will be for general healthcare services. As a result, attendant adverse risks and impacts related to infection control and transmission are not anticipated to substantiate incremental risks and at the Substantial level, as with the Eswatini COVID-19 Emergency Response Project which is being implemented in parallel but on a short-term implementation horizon.



89. **The proposed technical assistance (Component 1) activities are anticipated to have generally positive environmental benefits related to HCWM and methane emissions control.** Downstream adverse risks and impact could be anticipated related to implementation of HCWM and emissions control measures as well as proposed public-private partnership measures. This risks and impacts would be expected to be consistent with activities directly supported under the project and are therefore moderate.

90. **Finally, the project does not involve any significant social impacts that could harm communities and individuals or their livelihoods.** The key social concerns may include labor and working conditions (including minimal labor influx, gender-based violence [GBV], and sexual exploitation and abuse [SEA]) and community health and safety (including workers' community interactions and so on). The other potential social risks that need to be addressed relate to the E&S risk management capacity of both national and local health facility levels which is low, implementation and reporting of the relevant concerns, budget allocation for the implementation of E&S risk management measures, and exclusion of disadvantaged and vulnerable groups of people and individuals.

91. **Eight standards of the World Bank ESF apply to this project.** The E&S risk management instruments to manage and mitigate the project impacts and risks that were prepared include the Environmental and Social Management Framework (ESMF) and the Resettlement Policy Framework (RPF). These assess potential E&S risks and impacts and guide the preparation of appropriate site-specific plans during project implementation. The other tools/instruments include the Labor Management Procedures (LMP) as part of the ESMF, the Stakeholder Engagement Plan (SEP), and the ESCP. The ESMF, RPF, LMP, and SEP were consulted on December 12 and 17, 2019. These E&S instruments were publicly disclosed initially on March 15 and April 6, 2020, and redisclosed prior to negotiations on May 7, 2020. To mitigate against environmental risks related to management and disposal of healthcare waste, this project (P168564) will leverage the Eswatini COVID-19 Emergency Response Project) by adopting and implementing an Infection Control and Waste Management Plan (ICWMP) to be prepared under the project (P173883). For details, refer to Annex 1.

92. The project will build upon existing citizen engagement and grievance redress mechanisms (GRMs) already used by the MOH which includes both provider-triggered and beneficiary-triggered GRMs. Currently, members of the public can register their complaints on any health issue affecting them and their communities through a short message system (SMS) sent to a code 2020 anonymously at no cost to them. This is the beneficiary-triggered GRM while on the other hand, the same system is auto collecting beneficiary feedback from all who visit health facilities that are running on the CMIS asking them to rate the care they received on both clinical and non-clinical aspects and to register any grievance they might have on the health care received for that visit. Furthermore, the SEP for the project expands on other traditional grievance redress platforms that are open to beneficiaries to bring their complaints to the attention of the government authorities. These includes toll-free lines to the MOH, e-mail communication through the government website and community health forums where local community members voice out their concerns, and these get actioned by the health managers for those affected health facilities. The SEP also provides more details on how the GRM will be implemented and monitored to close the feedback loop throughout the life of the project. Feedback to clients at health facility level will be provided through health talks, media announcements, community health days, and directly to beneficiaries in cases of complaints affecting individual beneficiaries.



93. **Citizens engagement.** The project will conduct citizens engagement using a three-pronged approach:

- (a) **Inform the community about the project, its objectives, and importance.** The community will be informed about the project through free text alerts and community gatherings where information on the project and its objective will be provided by the RHMTs using different community platforms such as local chieftains and village health committee monthly meetings or disseminated through the CHVs or PHC staff.
- (b) **Consult to understand the community's needs.** During the start of the project, a local firm will be hired to conduct focus group discussions with the community to understand their socio-cultural norms in health care-seeking behavior, optimal nutrition practices, and quality of health services available to the community. This consultation will also explore the community needs and requirements. Additionally, on a routine basis, the CHVs through the group sessions will also work closely with the PHC staff to address these barriers and monitor uptake of services.
- (c) Feedback, grievance, and inputs from the community will be undertaken through the project grievance mechanisms, through RHMTs' community consultations, community health volunteer outreach sessions with support groups, and routine facility-based client satisfaction surveys. The objective is to facilitate continuous learning, improve project monitoring, and improve project outcomes. This information will be collated by the PIU who will work with the RHMT to analyze this information and close the feedback loop through their monthly review meetings.

Climate and Disaster Risks

94. The World Bank's Climate and Disaster Risk Screening Tool was used to assess the short- and long-term climate and disaster risks to the project. Eswatini is highly vulnerable to the impacts of climate change however the risk to impact on project activities is rated Moderate. Project location and target beneficiaries are vulnerable to climate-related shocks, including floods, droughts, and extreme temperature. Climate change is expected to result in increases in monthly maximum temperature, which are expected to be 1.2°C to 2.4°C above the historical mean. Similarly, the number of hot days (temperature above 35°C) is expected to rise by as many as 24.4 days by 2040–59 while the number of summer days (maximum temperature above 25 degrees Celsius) are expected to begin earlier and increase by at least 25 days, possibly by as much as 59 days by 2040–59. Eswatini has recently been affected by large variations in rainfall as well as recurring drought events. Precipitation patterns are expected to have increased variability with an increase in rainfall intensity. Annual precipitation is expected to decrease by 36 mm in 2040–59. The project contains a CERC which could be triggered to address the impacts of such extreme events.

95. The previously identified exposure to climate and disaster risks are predicted to affect the project's target population. More than 75 percent of Eswatini's population lives in rural areas, vulnerable to risk of climate change. Negative health impacts come from extreme climate events, such as heat waves, hurricanes/storms, floods, and droughts. Consequences of severe, recent droughts resulted in 25 percent of the population being vulnerable and acknowledged as food and water insecure. The regions with the highest prevalence of food insecurity are Lubombo and Shiselweni, the areas most affected by the drought. Gradual changes of climate affecting water, food, and air quality also have negative influence on



human health. Some of the possible direct threats that climate change could pose on human health in Eswatini include morbidity due to thermal stress, vector-borne infectious diseases, diarrhea, and malnutrition. According to the WHO, outbreak of human diseases such as malaria, dengue, diarrhea, cholera, typhoid, and other vector-borne diseases coincide with the occurrence of extreme climate events such as droughts and floods.

Climate Co-benefits Activities

96. **This project will contribute to climate adaptation.** Strengthening PHC and providing services in communities enhances resilience to climate-related extreme events such as flooding and wildfire which can isolate harder to reach vulnerable populations in rural areas. This project specifically targets climate vulnerable groups including women and children. Subcomponent 1.1 and Component 2 will also enhance the climate resilience of the health sector by developing and delivering training focused on climate and health for health workers and CHVs and through the SBCC modules. Subcomponent 1.2 makes the system more efficient and resilient by reducing the need for paper record keeping which can easily be lost during disaster or times of population migration. Digital health systems are being developed to provide a system that is more responsive to predicted changes in the epidemiology of climate-related health outcomes such as malaria, dengue, diarrhea, cholera, typhoid, and other vector-borne diseases as well as NCDs and maternal and child health issues. Under Subcomponent 1.3, strengthening facility, program, and regional capacity for quality service delivery and redesigning PHC-level delivery to expand the currently primarily communicable disease and maternal and child health-focused service coverage toward a high-quality, integrated care (including NCD and nutrition) further enhances population adaptation to climate change. Component 3 will also be part of the development and delivery of training for health leaders, which will include climate and health topics.

97. **This project will contribute to climate mitigation.** Strengthening PHC and community platforms moves care closer to home, reducing the need for more carbon-intensive hospital-based care. Subcomponent 1.2 that invests in tools and infrastructure also reduces the GHG emissions from the system through increased efficiency. Under this subcomponent, the purchase of goods and equipment, including blood pressure machines and other consumables will be conducted through a low-carbon procurement policy. This subcomponent will also implement the recommendations for energy efficiency audits of current facilities as a professional maintenance model and guidelines that will reduce the need to replace equipment and associated emissions implicit in the facilities are developed and implemented. The strengthening of environmental health, including facility-level sanitation in selected health facilities, through more efficient HCWM and support for centralized waste management and disposal of health care waste including the capture and combustion of fugitive methane will also reduce emissions.

Gender Tag

98. There are gender-based differences in the prevalence of stunting and wasting. Related to breastfeeding, boys are more likely to be exclusively or predominantly breastfed at five months compared to girls. Quality of complementary feeding, in terms of diversity, frequency, and minimum acceptable diets, is also slightly higher for boys. Related to child health, there are no significant gender related differences for immunization and care seeking behaviors during illnesses; however, restrictions on feeding during illness is more likely among girls than boys. There are also gender disparities in NCDs; while the prevalence of hypertension and diabetes is higher among women, men are less likely to seek care and



achieve disease control. The project will screen children for malnutrition and diseases and link them to preventive services and treatment. Outreach programs at the community level using CHVs will be organized and monitored to ensure that women, especially adolescents and young mothers, seek health services. Quality of care will be improved in health facilities to ensure that women, children, and adolescents receive care in a friendly and respectful environment. The SBCC supported by the project will also be tailored to ensure that gender-relevant messages are included and targeted to vulnerable groups such as pregnant women/mothers and adolescents, as well as targeted to men to the NCD gender gap.

V. GRIEVANCE REDRESS SERVICES

99. Communities and individuals who believe that they are adversely affected by a World Bank (WB) supported project may submit complaints to existing project-level grievance redress mechanisms or the WB's Grievance Redress Service (GRS). The GRS ensures that complaints received are promptly reviewed in order to address project-related concerns. Project affected communities and individuals may submit their complaint to the WB's independent Inspection Panel which determines whether harm occurred, or could occur, as a result of WB non-compliance with its policies and procedures. Complaints may be submitted at any time after concerns have been brought directly to the World Bank's attention, and World Bank Management has been given an opportunity to respond. For information on how to submit complaints to the World Bank's corporate Grievance Redress Service (GRS), please visit <http://www.worldbank.org/en/projects-operations/products-and-services/grievance-redress-service>. For information on how to submit complaints to the World Bank Inspection Panel, please visit www.inspectionpanel.org.

VI. KEY RISKS

100. **The overall risk rating is Moderate. The political and governance risk was rated Substantial,⁵¹ given the complex decision-making processes across the formal and traditional governance structures, affecting borrowing from international entities such as the World Bank Group.** Building on the lessons from the Performance and Learning Review of the CPS for FY2015–FY2018, as part of risk mitigation, the team engaged early on with the central agencies (MOF, MOEPD, and MOPS) during project development, and will involve political stakeholders (for example, engagements at the Cabinet level) to inform decision making. The project proposes to support leadership strengthening.

101. The macroeconomic risk was rated Substantial as the country's fiscal trajectory continues to be an area of concern. As revenue streams have reduced due to economic slowdown in the subregion,⁵² to curb the deficit, the Prime Minister announced fiscal consolidation measures to deal with specific issues concerning economic recovery. The proposed project will contribute to cost control by increasing the focus on the efficient use of resources to produce better outcomes and rationalizing capital investments in the sector.

⁵¹ This is consistent with the Systematic Country Diagnostic (No. 892010-SZ) and the CPIA rating.

⁵² Miscellaneous and unidentified revenue represented the largest share of non-tax revenues in 2016 (9.2 percent of GDP and 82.3 percent of non-tax revenues). The majority of this category comes from the Southern African Customs Union revenue-sharing agreement (OECD 2018). Eswatini's major source of revenue dropped by 18 percent from SZL 7.1 billion (2017/18) to SZL 5.8 billion (2018/19) and is set to decline further (Times of Swaziland 2018)



102. **The risk rating for institutional capacity for implementation and sustainability is Substantial.** Organizational, management, and capacity strengthening for the MOH and at all levels of the sector is required to support a steady reform process. Furthermore, structural rigidities beyond the MOH can affect project preparation and implementation, which require broader policy and technical engagements and sustained efforts to improve public sector performance. The MOH's more hands-on approach with project management and the gradual incorporation of roles/activities in the MOH's organization and work program and the proposed advisory bodies are some of the measures to mitigate this risk.



VII. RESULTS FRAMEWORK AND MONITORING

Results Framework

COUNTRY: Eswatini

Health System Strengthening for Human Capital Development in Eswatini

Project Development Objectives(s)

The project development objective is to improve the coverage and quality of key reproductive, maternal, neonatal, child and adolescent health (RMNCAH), nutrition and NCD services (hypertension and diabetes) in Eswatini.

Project Development Objective Indicators

Indicator Name	PBC	Baseline	Intermediate Targets				End Target
			1	2	3	4	
Improved coverage of key RMNCAH, nutrition and NCD services							
Percentage of patients diagnosed with hypertension and diabetes who are managed at the PHC level (Percentage)		0.00	30.00	50.00	60.00	70.00	80.00
Percentage of children under 5 years of age provided with a basic package of nutrition specific services (Percentage)		0.00	30.00	60.00	70.00	80.00	85.00
Percentage of adolescents served or reached with quality sexual and reproductive health services (Percentage)		0.00	30.00	40.00	50.00	60.00	70.00
Improved quality of RMNCAH, nutrition and NCD services							



Indicator Name	PBC	Baseline	Intermediate Targets				End Target
			1	2	3	4	
Percentage of pregnant women receiving at least 4 ANC visits that meet defined quality standards (Percentage)		0.00	40.00	45.00	50.00	55.00	60.00
Percentage of health facilities that meet a minimum standard of quality as measured by a Health Facility Quality Index (Percentage)		0.00	20.00	40.00	50.00	70.00	80.00

Intermediate Results Indicators by Components

Indicator Name	PBC	Baseline	Intermediate Targets				End Target
			1	2	3	4	
Improve health service delivery to increase coverage and quality of health services							
People who have received essential health, nutrition, and population (HNP) services (CRI, Number)		0.00	99,662.00	101,734.00	103,814.00	105,862.00	107,882.00
Number of women and children who have received basic nutrition services (CRI, Number)		0.00	74,000.00	76,000.00	78,000.00	80,000.00	82,000.00
Number of deliveries attended by skilled health personnel (CRI, Number)		0.00	25,662.00	25,734.00	25,814.00	25,862.00	25,882.00
Percentage of women receiving post-partum check-up within 7-14 days		84.70	86.00	88.00	90.00	90.00	90.00



Indicator Name	PBC	Baseline	Intermediate Targets				End Target
			1	2	3	4	
(Percentage)							
Number of people screened and diagnosed with hypertension (disaggregated by sex) (Number)		22,160.00	22,500.00	30,234.00	30,500.00	31,000.00	31,230.00
Number of people screened and diagnosed with diabetes (disaggregated by sex) (Number)		1,682.00	2,000.00	6,500.00	7,000.00	7,500.00	8,000.00
Number of HCWs trained to deliver high-quality RMCAH, nutrition and NCD services (Number)		0.00	70.00	150.00	150.00	150.00	150.00
Number of facilities using eLMIS for supply management (Number)		0.00	3.00	4.00	8.00	12.00	20.00
Proportion of health facilities using CMIS for routine reporting (Percentage)		54.00	60.00	65.00	70.00	75.00	80.00
Percentage of health facilities that have received at least one comprehensive quality assessment applying the appropriate checklist and protocol, and were provided adequate feedback from RHMT (Percentage)		0.00	20.00	60.00	70.00	80.00	90.00
Satisfaction rates of clients with the quality of maternal, child and adolescent health services (Percentage)		50.00	55.00	60.00	65.00	70.00	75.00
Increase community demand for RMNCAH, nutrition and NCD services							



Indicator Name	PBC	Baseline	Intermediate Targets				End Target
			1	2	3	4	
Number of community health workers including RHMs trained to conduct community sensitization & outreach on RMNCAH, Nutrition and NCD services (Number)		0.00	500.00	2,000.00	2,000.00	500.00	5,000.00
Number of mothers and adolescents reached with RMNCAH, Nutrition and NCDs specific communication as part of targeted SBCC interventions (Number)		0.00	74,000.00	76,000.00	78,000.00	80,000.00	82,000.00
Number of client-based digital applications' users (Number)		0.00	3,700.00	7,600.00	15,600.00	24,000.00	41,000.00
Strengthen MOH's stewardship capacity to manage health & nutrition services and project activities							
Number of updated regulations, policies and strategies for MOH (Number)		0.00	2.00	2.00	4.00	4.00	4.00
Number of MOH staff trained (technical and leadership) (Number)		0.00	20.00	20.00	20.00	20.00	20.00
Percentage of health facilities with a protocol to respond to gender-based violence in place (Percentage)		18.00	23.00	28.00	33.00	38.00	43.00



Monitoring & Evaluation Plan: PDO Indicators

Indicator Name	Definition/Description	Frequency	Datasource	Methodology for Data Collection	Responsibility for Data Collection
Percentage of patients diagnosed with hypertension and diabetes who are managed at the PHC level	Numerator: Number of patients (disaggregated by sex) who are diagnosed with hypertension plus number of patients who are diagnosed with diabetes who were managed as per protocol at PHC level Denominator: Total number of patients (disaggregated by sex) diagnosed with; i) hypertension and ii) diabetes	Quarterly	CMIS	Routine	MOH- HMIS unit
Percentage of children under 5 years of age provided with a basic package of nutrition specific services	Numerator: Number of children under 5 years (disaggregated by sex) who were provided with basic package of nutrition services by a community health care worker. Denominator: Total number of children under 5 years (disaggregated by sex) seen by a community health care worker	Quarterly	CMIS	Routine	MOH - HMIS unit



Percentage of adolescents served or reached with quality sexual and reproductive health services	Numerator: Number of adolescents served or reached with quality sexual and reproductive health services offered in the target/catchment facilities. Denominator: Total number of adolescents in the population of the target/catchment facilities.	Quarterly	CMIS	Routine	MOH-HMIS unit
Percentage of pregnant women receiving at least 4 ANC visits that meet defined quality standards	Numerator: Number of pregnant women seen at a health facility who have four or more ANC visits recorded that meet defined standards including assessment for high risk pregnancy at each visit Denominator: Total number of pregnant women seen for ANC at a health facility	Monthly	CMIS	Routine	MOH - HMIS unit
Percentage of health facilities that meet a minimum standard of quality as measured by a Health Facility Quality Index	Numerator: Number of facilities that meet a minimum standard of quality as measured by a Health Facility Quality Index Denominator: Total number of facilities assessed based on the minimum standard of	Semi-annually	Health Facility Quality Assessment reports; Client Satisfaction Feedback Mechanism	Surveys	National Quality Management Program (NQMP)



	quality				
	NB: The Health Facility Quality Index is a combined average score from a facility quality of care assessment and the client satisfaction score for the same facility. Baseline will be available in year 2 of the project.				

Monitoring & Evaluation Plan: Intermediate Results Indicators

Indicator Name	Definition/Description	Frequency	Datasource	Methodology for Data Collection	Responsibility for Data Collection
People who have received essential health, nutrition, and population (HNP) services		Monthly	CMIS	Routine	MOH - HMIS unit
Number of women and children who have received basic nutrition services					
Number of deliveries attended by skilled health personnel					
Percentage of women receiving post-partum check-up within 7-14 days	Numerator: Number of women receiving post-partum check-up within 7-14 days after partum. Denominator: Total number of births	Monthly	CMIS Baseline figure from: Provision of postpartum care to women giving birth in health	Routine	MOH - HMIS unit



			facilities in sub-Saharan Africa: A cross-sectional study using Demographic and Health Survey data from 33 countries, 2019		
Number of people screened and diagnosed with hypertension (disaggregated by sex)	Numerator: refers to the total number of persons (disaggregated by sex) who were screened and diagnosed with hypertension Denominator: N/A	Monthly	CMIS	Routine	MOH - HMIS unit
Number of people screened and diagnosed with diabetes (disaggregated by sex)	Numerator: refers to the total number of persons (disaggregated by sex) who were screened and diagnosed with diabetes Denominator: N/A	Monthly	CMIS Baseline from 2018 NCD Annual report	Routine	MOH-HMIS unit
Number of HCWs trained to deliver high-quality RMCAH, nutrition and NCD services	Numerator: Number of HCWs trained each year under the project (not cumulative)	Monthly	CMIS	Routine	MOH - HMIS



	Denominator: N/A				
Number of facilities using eLMIS for supply management	Numerator: Number of facilities using eLMIS for supply management Denominator: N/A	Quarterly	eLMIS roll-out implementation report	customized	Central Medical Stores (CMS)
Proportion of health facilities using CMIS for routine reporting	Numerator: Number of health facilities using CMIS for routine reporting Denominator: Total number of health facilities NB: baseline determined using denominator of 327 health facilities as reported in SARA 2017	Quarterly	CMIS roll-out implementation report	Customized	MOH-HMIS
Percentage of health facilities that have received at least one comprehensive quality assessment applying the appropriate checklist and protocol, and were provided adequate feedback from RHMT	Numerator: Number of Health facilities that have received at least one comprehensive quality assessment applying the appropriate checklist and received feedback Denominator: Total number of health facilities that received a comprehensive quality assessment applying the appropriate checklist	Monthly	Regional Quality Assessment Reports	Assessment	RHMTs & NQMP



Satisfaction rates of clients with the quality of maternal, child and adolescent health services	Numerator: Number of patients reporting to have been satisfied with health services they received during a visit to a health facility Denominator: Total number of patients who responded on the patient feedback system	Quarterly	Client Satisfaction Feedback Mechanism (CMIS)	Survey (Client Satisfaction Feedback Mechanism is SMS based and sends short message to all clients who visits a CMIS health facility within an hour after the visit to provide feedback on their experience with the facility and care provided.)	National Quality Management Program
Number of community health workers including RHMs trained to conduct community sensitization & outreach on RMNCAH, Nutrition and NCD services	Number of community health workers including RHMs trained to conduct community sensitization & outreach on RMNCAH, Nutrition and NCD services	Quarterly	Community Health Workers Training Records	Routine	MOH-Rural Health Motivators Program
Number of mothers and adolescents reached with RMNCAH, Nutrition and NCDs specific communication as part of targeted SBCC interventions	Numerator: Number of mothers and adolescents reached with RMNCAH, Nutrition and NCDs specific communication as part of targeted SBCC interventions Denominator: N/A	Quarterly	Community Health Workers Service Coverage reports	Routine	MOH-Rural Health Motivators Program
Number of client-based digital applications' users	Numerator: Number of client-based digital applications' users Denominator: N/A	Quarterly	Digital Applications System Downloads Tracker	Routine	MOH-Rural Health Motivators Program



	NB: Digital application uptake is assumed at 5% of the reached mothers and adolescents in year one rising to 50% by year 5.				
Number of updated regulations, policies and strategies for MOH	Numerator: Number of updated regulations, policies and strategies for MOH Denominator: N/A NB: In total, about 7 strategic documents will be developed/finalized through resources provided under this project	Quarterly	MOH-Directorate records	Routine	MOH-SMT
Number of MOH staff trained (technical and leadership)	Numerator: Number of MOH staff trained (technical and leadership) Denominator: N/A NB: Trainings for MOH staff are targeting 20 participants in 3 different trainings and most will participate in all three.	Quarterly	MOH HQ-Training Records	Routine	MOH-Training Office
Percentage of health facilities with a protocol to respond to gender-based violence in place	Numerator: Number of health facilities that have a protocol to respond to	Quarterly	Health Facility Supportive	Checklist	RHMTs



	gender-based violence in place		Supervision Records		
	Denominator: Total number of health facilities assessed				
	NB: Target is 5% increase annually to be aligned with roll-out plans for NCDs and Nutrition services as well as CMIS				



ANNEX 1: Implementation Arrangements and Support Plan

ANNEX 1A. Strategy and Approach for Implementation Support

1. A strategy for implementation support has been developed based on lessons learned from previous World Bank experience in the Southern Africa subregion and past and ongoing projects in Eswatini and based on this project's specific challenges and risks. The MOH, with support from a PIU, is responsible for project implementation. The strategy for implementation support aims to provide continuous targeted support and technical advice from the World Bank to the MOH and the PIU on implementation and ensuring compliance to World Bank guidelines and procedures on fiduciary and E&S aspects.

2. The project will require substantial support from the World Bank's task team in the early years of implementation. The World Bank will work with the MOH and the PIU to ensure that effective coordination is established. The World Bank will work with the MOH and the PIU and agree on regular reporting formats. The reports will include updates on technical and financial progress and on monitoring indicators. The World Bank will also advise the MOH and PIU on environmental due diligence to ensure that the ESF tools, prepared for the project, are adhered to. The World Bank will work with the Borrower to identify underperforming areas (if any) and address any constraints. Additional implementation support will include monitoring changes in risks and compliance with legal covenants.

3. The World Bank's implementation support will broadly consist of (see Table 1.1)

- (a) Institutional capacity-building activities, covering technical, fiduciary, and social and environmental dimensions;
- (b) Provision of technical advice and implementation support geared to the attainment of the PDOs, including support to the MOH to improve organizational functionality and strengthen stewardship;
- (c) Ongoing monitoring of implementation progress, including regularly reviewing key outcome and intermediate indicators;
- (d) Review of financial and progress reports prepared by the Borrower and audit reports prepared by internal and external auditors;
- (e) Monitoring of risks and identifying the corresponding mitigation measures; and
- (f) Close coordination with other DPs to leverage resources to ensure coordination of efforts and avoid duplication.

4. The World Bank's implementation support team will consist of a Health, Nutrition, and Population Global Practice Task Team Leader and field-based and headquarters-based staff in the areas of health and



nutrition, FM, procurement, environmental and social, and operations. Expert consultants may also support the project in specific subject areas. Task team members will be responsible for supporting the implementation of project-specific elements in their areas of expertise (Table 1.2).

Table 1.1. Implementation Support Plan

Time	Focus	Skills Needed	Resource Estimate
First 12 months	Technical advice to support project implementation (procurement, M&E, FM)	Technical, fiduciary, operational, environment, and social	At least two implementation support visits by technical specialists focused on planning, capacity building, TA, and monitoring Two implementation support visits by E&S specialists focused on capacity building and reviewing/strengthening effectiveness of redress mechanism
12–24 months	Continued technical support for project implementation Implementation monitoring	Technical, fiduciary, environment, and social	Two implementation support visits by technical and fiduciary specialists focused on fiduciary and implementation support Two implementation support visits by social/environmental specialists focused on strengthening local capacity, and implementation support
Implementation midterm	Implementation progress review and identification of necessary midcourse adjustments	Technical, fiduciary, environment, social, and operational	One implementation support visit including technical, fiduciary, social, environment, M&E, and operational specialists
24–60 months	Implementation M&E Technical advice to support program implementation and closing	Technical and fiduciary	Two implementation support visits by technical and fiduciary specialists focused on implementation and fiduciary support Two implementation support visits by social/environmental specialist focused on strengthening local capacity, and implementation support

Table 1.2. Task Team Skills Mix Requirements for Implementation Support

Skills Needed	Estimated Number of Staff Weeks Per Fiscal Year
Program Management (Task Team Leader)	16
Health Specialist	10
Nutrition Specialist	4
M&E Specialist	6
FM Specialist	3
Procurement Specialist	3



Environmental Specialist	4
Social Specialist	4
Operations Officer	6
Administrative Support	2

B. Financial Management and Disbursement Arrangements

1. The World Bank conducted an FM assessment of the MOH as the implementing entity and as required by the World Bank's policy on Investment Project Financing. The main objective of the assessment, which included a review of the budgeting, accounting, internal controls, flow of funds, financial reporting, auditing arrangements at the MOH and completion of FM assessment questionnaire by the financial controller and principal accountant of the department, was to ensure that acceptable FM arrangements are in place for the implementation of the project.

2. Acceptable FM arrangements ensure that

- (a) The funds are used only for the intended purposes in an efficient and economical way,
- (b) All transactions and balances are correctly recorded to support preparation of regular and reliable financial statements that are subject to auditing arrangements acceptable to the World Bank; and
- (c) Internal controls are considered capable of safeguarding the agencies' assets.

3. The overall conclusion of the FM assessment is that the project's FM arrangements have an overall risk rating of Moderate, which satisfies the World Bank's minimum requirements under OP/BP 10.00. **except for** the issues referred to in the FM Action Plan.

4. **Risk assessment and mitigation.** Table 1.3 shows the results of the risk assessment from the risk rating summary. This identifies the key risks project management may face in achieving project objectives and provides a basis for determining how management should address these risks.

Table 1.3. FM Risk Mitigation Assessment Risk Rating

Risk	Risk Rating	Risk Mitigating Measures Incorporated into the Project Design	Conditions of Negotiations, Board or Effectiveness (Yes or No)	Residual Risk Rating
Inherent risk				
Country level The country is advancing in its public financial management (PFM) reforms with the new PFMA enacted in 2017. Reforms in the	M		No	M



Risk	Risk Rating	Risk Mitigating Measures Incorporated into the Project Design	Conditions of Negotiations, Board or Effectiveness (Yes or No)	Residual Risk Rating
Integrated Financial Management Information System (IFMIS) are also under way with the support from the European Union.				
Entity Level The entities may not be able to meet the FM requirements due to lack of FM capacity.	M	A qualified and experienced FMS will be hired in the PIU to handle the fiduciary matters (FM, disbursement) of the project.	By no later than one (1) month after the Effective Date, the Borrower shall appoint qualified and experienced project FMS.	M
Project level	M			M
Control risk				
Budgeting The implementing agency may not prepare realistic annual plans and budget	M	The budget preparation will be based on approved annual workplans which will be subjected to vigorous reviews and oversight will be done by the World Bank's team.		M
Accounting No risk identified; the project will use the accounting system from the recently closed project which has proven to be adequate.				
Internal control Risk that some internal control procedures may not be observed by the project staff.	M	For smooth implementation of the project, the ministry's internal auditor will be required to include the project in its annual internal audit plans.		M
Funds flow	n.a.		No	n.a.
Financial Reporting The project may not be able to produce periodic reports to monitor and effectively	M	The FMS will be fully dedicated to the FM of the project, including financial		M



Risk	Risk Rating	Risk Mitigating Measures Incorporated into the Project Design	Conditions of Negotiations, Board or Effectiveness (Yes or No)	Residual Risk Rating
manage the project.		reporting. The FMS will be fully responsible for producing IFRs for timely submissions.		
Auditing No risk identified; the MOF was in compliance with the audit submissions in the recently closed project.				
Overall risk	M	Based on the risks identified and proposed mitigations, the FM risk will be Moderate .		M

H (High), S (Substantial), M (Moderate), and L (Low)

11. **Major strength.** The MOH is familiar with the World Bank's requirements based on the experiences of the recently closed project.

5. **Weaknesses and action plan.** The proposed FM staff may be unfamiliar with specific financial and disbursement arrangement of the World Bank. Onboard and periodic trainings will be offered to mitigate this risk.

6. **Budgeting.** The annual budget will be prepared based on approved work plans which will be reviewed and approved by the technical team in the World Bank. Monitoring of the budget will occur through the reviews of the management reports and the interim unaudited financial reports (IFRs) which will be submitted to the World Bank on a quarterly basis.

7. **Accounting.** The project accounting system will be based on government accounting software called 'Entire'. The experience with the closed project has proven that the system can produce project annual financial statements, IFRs, and other reports necessary to monitor and effectively manage the project.

8. **Staffing.** The overall responsibility for project FM rests with the ministry's Financial Controller who is supported by the principal accountant. The project FMS who will be recruited by effectiveness will report to the financial controller and the project coordinator. The project accountant will act as the focal person during the project preparation, until such time the FMS is recruited and fully onboarded. The principal accountant will be seconded to the PIU for the entire project implementation from the MOH pool of accountants. This arrangement will not compromise the MOH staffing arrangements.

Internal Control and Internal Auditing Arrangement



9. **Internal controls.** The project's internal controls will be based on the GOE's PFMA of 2017 and supplemented by the Operational Manual when the need arises. The project accountant will assist in the preparation and review on the Operations Manual until the recruitment of the FMS.

10. **Internal audit.** The internal audit unit based at MOF will be responsible for performing internal audit functions across the entire GOE. Considering limited capacity of this unit, and involvement of various spending units, there is a need to put in place a strong internal control mechanism and quality control assurance system at all levels. Therefore, the project will use a qualified private auditor for the internal audit with relevant experience with donor-financed projects within four months after the project effectiveness.

11. **Financial reporting.** The project will produce on a regular basis required financial reports to monitor and effectively manage the project. Unaudited IFRs will be produced on a quarterly basis and submitted to the World Bank 45 days after the financial quarter. These reports should consist of the following:

- (a) A narrative summary of the project implementation highlights
- (b) Sources and uses of funds
- (c) Uses of funds by project component/activity- both actual and cumulative
- (d) The Designated Account activity statement
- (e) Summary of payments made for contracts subject to the World Bank's prior review
- (f) Summary of payments made for contracts not subject to the World Bank's prior review

12. The project accounting system can produce quarterly reports. The reports will be submitted to the MOF with relevant supporting documentation within 30 days of the end of the reporting quarter. Final review and submission of the reports to the World Bank by the MOF is within 45 days of the end of the reporting period.

13. PST will also produce annual project financial statements, which will consist of the following:

- A Statement of Sources and Uses of Funds/Cash Receipts and Payments that recognizes all cash receipts, cash payments and cash balances controlled by the entity for this project and separately identifies payments by third parties on behalf of the agency.
- The Accounting Policies Adopted and Explanatory Notes should be presented in a systematic manner with items on Statement of Cash Receipts and Payments being cross-referenced to any related information in the notes. Examples of this information include a summary of fixed assets by category of assets.

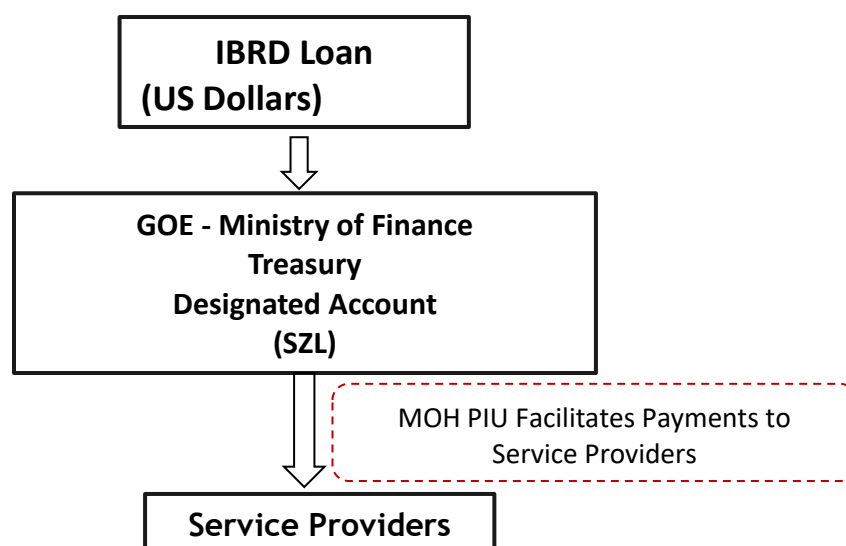


- A Management Assertion that IBRD funds have been expended in accordance with the intended purposes as specified in the relevant World Bank Loan Agreement.

Funds Flow and Disbursement Arrangement

14. **Flow of funds.** The agreed funds flow diagram is presented in figure 1.1 and should be viewed in the context of the funds flow arrangement as well disbursement arrangements described in the following paragraphs.

Figure 1.1. Flow of Funds



15. **Funds flow.** Upon the signing of the Loan Agreement, the World Bank will open a loan account in its books, in the name of the GOE. Funds will flow from the World Bank (loan account) into the US dollar Designated Account to be opened by the GOE at the Central Bank. A secondary Designated Account will be opened and maintained in the local currency by the MOF to receive funds from the Central Bank and finance activities of all the project components.

Banking Arrangements

16. **Designated Account.** Two Designated Accounts, denominated in US dollars, are to be opened at the Central Bank of Eswatini; one for components 1, 2 and 3, and another for component 4 (CERC). Disbursements from the World Bank loan will be deposited in these accounts. A local currency project account will be maintained by the MOF to finance project activities.

17. **Disbursement arrangements.** The project will use the Advance disbursement method whereby withdrawals from the loan account will be deposited in the Designated Account for payment of the World



Bank-financed eligible expenditures. Disbursements from the loan account will be based on quarterly IFR documentation to be prepared and submitted by the MOH to the MOF. The IFRs will be supported will be supported by documentations as may be required by MOF. The submission will be within 30 days of the end of the reporting quarter. For withdrawal from the loan account, MOF will be responsible for submitting withdrawal applications supported by IFRs, within 45 days of the end of each reporting period. The GOE will also have the option of using: (a) the Direct Payment disbursement method involving direct payments from the loan account on behalf of the GOE to the suppliers of goods and services that have a value above set threshold, (b) the Reimbursement disbursement method, whereby the GOE makes payments for the World Bank eligible expenditures and submits withdrawal applications for reimbursement, and (c) the Special Commitment method whereby the World Bank, at the request of the GOE, will issue special commitments to suppliers of the goods under the World Bank-financed components. Upon effectiveness of the Loan Agreement and submission of a withdrawal application, the World Bank will disburse an amount equivalent to six months expenditure into the Designated Account. Subsequent disbursements will be based on a six- monthly estimated expenditure, considering the balance in the DA at the end of the reporting period.

18. **External audit.** The project financial statements will be audited by the Office of the Auditor-General of Swaziland in accordance with International Standards on Auditing promulgated by the International Organization of the Supreme Audit Institutions. The audit report, together with the Auditor General's Management Letter and management response, will be submitted to the World Bank within six months after the financial year-end, that is, September 30 each year.

19. The Auditor General will be required to express a single opinion on the project financial statements. These will include financial information of activities implemented under all project components. In addition, a detailed management letter containing the auditor's assessment of the internal controls, accounting system and compliance with financial covenants in the IBRD Loan Agreement, suggestions for improvement, and management's response to the auditor's Management Letter will be prepared and submitted to the management for follow-up actions.

20. Project governance and accountability. No matters of governance and accountability came to light during the assessment. However, the project will have adequate FM in place to ensure that funds are used for the purposes intended and to prevent material errors and fraud. The World Bank will review adequacy of project FM during implementation review missions, and the GOE will ensure that adequate FM is maintained through the implementation of the project.

Conditionality

- (a) **Staffing.** By no later than one (1) month after the Effective Date, the Borrower shall appoint a qualified and experienced project FMS to be seconded by the principal accountant.
- (b) **Action plan.** To establish an acceptable control environment and to mitigate FM risks, the following measures should be taken by the due dates as indicated in the FM action plan in Table 1.4.



Table 1.4. Action and Completion Date

	Action	Responsibility	Completion Date
1	Appointment of qualified and experienced project FMS for the PIU	MOH	Effectiveness
2	Secondment of principal accountant to PIU	MOF and MOH	Effectiveness

(c) **Supervision plan.** FM supervision will be carried out by the FMS once a year in line with the moderate risk rating. The FMS will also

(i) Review the quarterly IFRs; and

(ii) Review the Audit Reports and Management Letters from the external auditors and follow up on material accountability issues by engaging with the task team leader, client, and/or auditors.

Overall Conclusion

21. The conclusion of the FM assessment is that the project's FM arrangements have an overall risk rating of Moderate, which satisfies the World Bank's minimum requirements under Policy/Directives: Investment Project Financing.



C. Procurement

1. The GOE, through the Office of the Prime Minister, setup an inter-ministerial task team in 2017 to look at the state of service delivery in the health sector including procurement. The team identified some weaknesses and made recommendations and corrections which were approved by the Cabinet, summarized in the Table 1.5.

Table 1.5. Procurement Assessment Findings and Recommendations

Challenge	Recommendations	Actions and Responsibility	Cabinet Decision	Means of Verification	Time Frame	Current Status/Support Required
		Responsible agencies for the actions include the MOH Procurement Unit, PS in the MOF, Head of Financial Marketing Central Bank, Accountant General and PS of Ministry of Labor and Social Security				The Procurement Unit needs basic office equipment; (filing cabinet, shredder, computers, internet access for STEP, stationery, projector, binding machine) US\$35,000. Electronic document storage (server - currently all soft copies are stored only in laptops) US\$8,000. Vehicle US\$40,000). The unit uses furniture from the previous project. The unit does not have a receptionist – all telephones lines connect to the CMS. It is difficult



Challenge	Recommendations	Actions and Responsibility	Cabinet Decision	Means of Verification	Time Frame	Current Status/Support Required
						to contact the unit by telephone.
Inadequate procurement planning, contract management, performance measurement, internal controls and visibility into value of procurement. Weak procurement management information system which limits information availability and accessibility and use to make procurement and supply decisions such as order quantities and delivery management	Prepare annual Procurement Plan (to be revised quarterly) using inputs from user departments	Develop annual Procurement Plan (to be revised quarterly) using inputs from user departments	APPROVED	Availability of current Procurement Plan and status of key performance indicators	December 2020	The unit has started training user departments on development of annual Procurement Plans.
	Carry out contract management function	Assign the responsibility for procurement contract management to one of the procurement officers	APPROVED	Contract management functions undertaken by a designated officer	March 2021	Basic Excel based contract management dashboard was developed by the Clinton Health Access Initiative; contracts for pharmaceuticals were populated in the spreadsheet. It is manually populated, not centralized, and has no visibility. It is updated per officer. Users have not been trained Training of officers required in the following: World Bank Procurement Procedures



Challenge	Recommendations	Actions and Responsibility	Cabinet Decision	Means of Verification	Time Frame	Current Status/Support Required
						• Procurement of goods (goods and equipment, consultancy services) • Contract management • Framework agreements • Strategic purchasing Training US\$40,000
	Align the MOH Procurement Strategy with the GOE Procurement Policy	▪ Develop and implement the MOH procurement strategy	APPROVED	Availability of MOH Procurement Strategy	March 2021	Procurement strategy not yet developed. Supply chain strategy was done; however, it places more emphasis on CMS procurement; TA to support the development of the procurement strategy US\$30,000
	Prepare procurement supply plans which will inform demand/orders to suppliers on time	▪ Prepare a procurement supply plan	APPROVED	Existence of procurement supply plans for each supplier	March 2021 Ongoing	To be done by the CMS and shared with the Procurement Unit. Not yet done. Training of officers on quantification is required (at the CMS and facilities) US\$20,000



D. Environmental and Social

1. To address any potential negative E&S impacts and risks of this health systems strengthening for human capital development project the following standards are relevant: ESS1 (Assessment and Management of Environmental and Social Risks and Impacts), ESS2 (Labor and Working Conditions), ESS3 (Resource Efficiency and Pollution Prevention and Management), ESS4 (Community Health and Safety), ESS5 (Land Acquisition, Restrictions on Land Use and Involuntary Resettlement), ESS6 (Biodiversity Conservation and Sustainable Management of Living Natural Resources), ESS8 (Cultural Heritage), and ESS10 (Stakeholder Engagement). An ESMF, an RPF, an LMP as part of the ESMF, and a SEP were consulted on December 12 and 17, 2019. These documents were disclosed both in-country and on the World Bank's website on May 7, 2020.

ESS1: Assessment and Management of Environmental and Social Risks and Impacts

2. This project has a relatively small footprint with limited construction activities (construction of the Operations Centre) and hence limited labor influx. Construction will be undertaken primarily on land already owned by benefiting institutions as project beneficiaries (for example, CMS). A project ESMF has been prepared to guide the development and implementation of site-specific E&S instruments, such as ESMPs.

ESS2: Labor and Working Conditions

3. The project footprint is relatively small and not likely to engage a significant number of site workers. Most of the labor will be already existing health workers in health facilities except for skilled local contract workers who will be required for the construction of the Operations Centre. Labor camps are not anticipated. The MOH has developed an LMP as part of the ESMF which illustrates types of workers to be engaged and their management in line with both ESS2 and national labor laws and regulations. Although labor influx is not anticipated, social impacts such as GBV, SEA, and communicable diseases for local communities cannot be ruled out. Before construction of the sanitation systems, a GBV risk assessment using the World Bank's GBV risk screening tool will be conducted, and management and mitigation of GBV/SEA risks will be integrated in the SEP and LMP. The ESMP will include labor related clauses, GBV risk management measures and preventive health measures. Contractors contracts will include specific clause of prioritizing recruitment of unskilled local labor and will consider social and environmental mitigation measures referring to the ESMPs and the World Bank's general Environment, Health and Safety Guidelines (EHSGs). Any civil works contracts will include the EHSGs, good international industry practice (GIIP), and industry standard codes of conduct that address OHS risks and include measures to prevent GBV/SEA. The LMP, including worker-specific GRM (for direct and contract workers), will be disclosed before Board approval.

4. It is anticipated that activities under Sub-component 1.2 will result in increased generation, handling and disposal of biomedical waste, posing OHS risk to health care workers. The ESMF includes requirements for inclusion of OHS measures/plans as part of the sub-project-specific ESMPs. The OHS measures will consider and align with the World Bank's general EHSGs, the industry sector EHSGs for Health Care Facilities, other GIIP, and the ICWMP to be prepared under the Eswatini COVID-19 Emergency Response Project.



ESS3: Resource Efficiency and Pollution Prevention and Management

5. The ESMF includes guidance with regard to waste management, water use, energy use, air quality, water quality, contamination of land and soil, and other risks associated with the construction and operation of the Operations Centre. Standard operating procedures will be prepared to address the treatment and disposal of e-waste and recommend how these will be applied and monitored. In addition, the design of the Operations Centre will be prepared in a way to contribute to climate co-benefits, including use of energy-efficient and sustainable technologies (for example, solar panel, insulation, refrigeration system, led lighting, etc.). The project will adopt and implement the Infection Control and Waste Management Plan (ICWMP) to be prepared under the Eswatini COVID-19 Emergency Response Project to mitigate and manage the risk of generation, handling and disposal of biomedical waste.

ESS4: Community Health and Safety

6. While the anticipated construction of the Operations Centre will have a small footprint, the MOH will consider the incremental risks of the public health and safety and potential exposure to operational accidents. It is anticipated that inadequacies in the management of health care waste might pose a risk to community health and safety. To mitigate this impact, ongoing monitoring of health care waste streams handling will be in place during project implementation. This risk will be mitigated through the implementation of the ICWMP to be prepared under the Eswatini COVID-19 Emergency Response Project. As part of sub-project-specific ESMPs, the project will evaluate site-specific community risks and impacts to the affected communities during the project life-cycle and will establish preventive and mitigation measures. As there might be minimal construction works, labor influx is not anticipated, and the project ESMF and its embedded LMP include clauses to avoid, minimize, manage, and mitigate any SEA and GBV risks. Additionally, GBV/SEA risk will be monitored throughout the project cycle. Construction of the Operations Centre may expose neighboring communities to noise and dust pollution, and increased traffic and road accidents (if there are unusual movements of transport of materials). While the construction works will take place in an existing footprint, the MOH will consider the incremental risks of the public's potential exposure to operational accidents.

ESS5: Land Acquisition, Restrictions on Land Use and Involuntary Resettlement

7. The project will involve the construction of an Operations Centre. This will be built on existing land owned by the MOH – CMS, and it is expected that there will be minimal and mostly temporary impacts (restricted land use and access). Should there be any restrictions or impacts on land and natural resource tenure and use, an RPF that has been prepared by this project as a precaution will provide guidance regarding resettlement principles, organizational arrangements and design criteria and the commitment to prepare a subsequent Resettlement Action Plan in cases where impacts cannot be avoided; this has been included in the ESCP. The RPF includes a GRM. The GRM will consider existing community, traditional dispute settlement mechanisms and availability of judicial systems. The cost associated for financing compensation and resettlement assistance, if any, shall be covered by the GOE.

ESS6 Biodiversity Conservation and Sustainable Management of Living Natural Resources



8. The project activities will not affect any natural or critical habitats. This ESMF will determine whether the establishment of the proposed Operations Centre at the CMS site will pose risks to any critical habitats or biodiversity, and the assessment will propose appropriate measures to address any such risks. The Operations Centre will be confined to the existing CMS compound which is in altered environments that do not contain any natural habitats. However, should the Operations Centre require more land use out of the CMS, necessary measures will be considered to safeguard biodiversity and living natural resources in a sustainable manner.

ESS8: Cultural Heritage

9. The project interventions will take place in the municipalities, and siting of the Operations Centre will be done in consultation with MOH officials and communities residing within the localities of the CMS. It is therefore unlikely that tangible or intangible cultural heritage will be affected. Nevertheless, the ESMP for the Operations Centre will identify measures to address risks and impacts on cultural heritage and develop a chance finds procedure—if applicable—to be implemented during installation/construction of infrastructure. Nonetheless, all construction contracts will incorporate a 'chance find' clause which will require contractors to stop works if any cultural properties are encountered during construction.

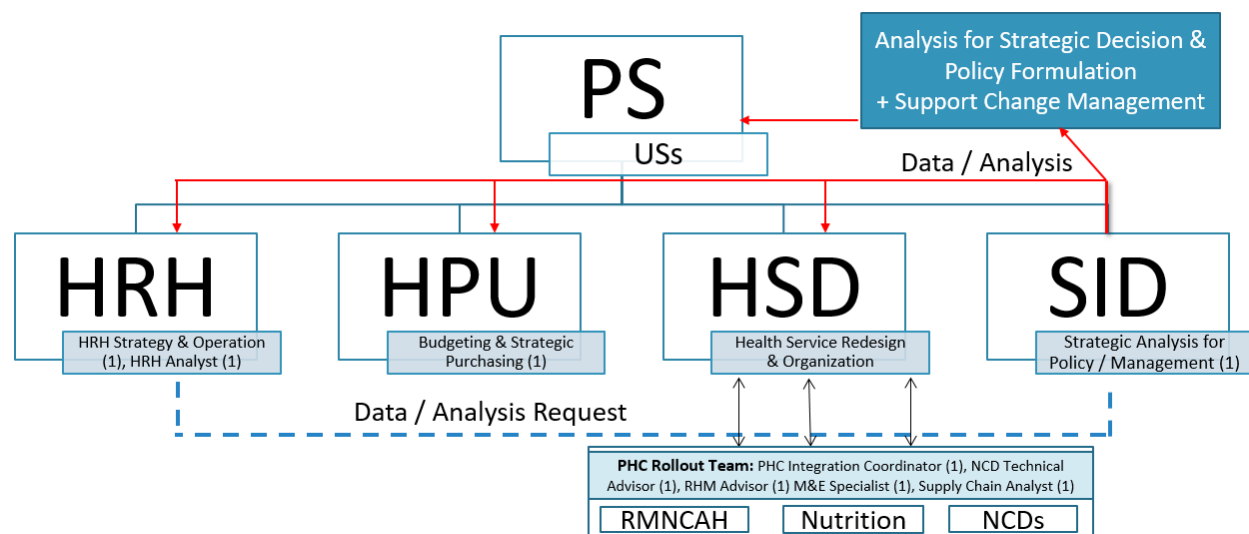
ESS10: Stakeholder Engagement and Information Disclosure

10. Stakeholder engagement is a key tool for E&S risk management and successful implementation of the project. A SEP has been prepared and was disclosed on May 7, 2020. The SEP identifies all key existing and potential stakeholders, and describes, among others, their level of influence and support to project and in its planning and implementation. It also describes means, timelines, and frequency of communication with each stakeholder/stakeholder group, grievance mechanisms to be deployed; and monitoring and reporting. Furthermore, the plan identifies all key existing and potential stakeholders and will engage in meaningful consultations (including health workers, project affected communities, women and youth groups, non-governmental organizations, patients, line ministries, community-based groups and disabled people's organizations, and other vulnerable and disadvantaged members of the communities) throughout the project life cycle. The MOH will provide stakeholders with accessible and inclusive GRM to raise issues and grievances, which will allow the MOH to receive, respond to, and facilitate resolution of concerns and manage grievances. The MOH will ensure that all stakeholder consultations are accessible and inclusive (in format and location) and that these consultations will be appropriate to the local context. The MOH will subsequently provide stakeholders with timely, relevant and understandable information in a culturally appropriate format. As part of the E&S assessment, the client will maintain and disclose documentation of stakeholder engagements, which will describe the stakeholders consulted, summary of issues discussed, and responses from the Borrower.



E. Additional Material on Implementation Arrangements

Figure 1.2. Strengthening MOH Stewardship and Technical Leadership - Support to Key MOH Departments



Note: HPU = Health Planning Unit; HSD = Health Services Directorate; SID = Strategic Information Department; US = Under Secretary

Table 1.6: Implementation Arrangements: Membership and Functions in the Policy and Technical Bodies

Structure/Committee	Membership	Function(s)
Interministerial Policy Advisory Group	- Chief Economist, MOED - PS, MOH - PS, MOEPD - PS, MOF - PS, MOPS - Project Coordinator	- Provide overall project oversight. - Make policy decisions to facilitate project implementation and coordination. - Meet at least once, every six months during project implementation.
Interministerial Technical Committee	- Director of Health Services, MOH (Chair) - Director of Management Services Division, MOPS - Director of Information, Communication and Technology, MICT - Human Capital Liaison - Project Coordinator (Secretariat) - Sector Officer for Health, MOF - Sector Officer for Health, MOEPD	- Provide technical oversight of project activities for Components 1 – 3. - Support coordination across project activities that require input from other ministries. - Meet monthly to review technical reports for submission to the Senior Management Team of the MOH.



ANNEX 2: Additional Material on Health System

1. **Hospital assessments show that inadequate strategic planning leads to service profiles that do not allow for efficient service delivery and compromise health outcomes. Lack of provider payment incentives leads to low value for money at the hospital level.** Most hospitals provide low-complexity care, which could be provided at lower costs at lower levels. Where high-complexity care is provided, performance outcomes are often modest (low occupancy and high mortality) due to inadequate service inputs and organization, which undermine patient trust. Low client expectations of high-complexity care contribute to demand for more costly overseas referrals.⁵³ Rigid historic line item budgeting (for example, public hospitals cannot reallocate resources across categories according to need; government subventions to mission hospitals have no performance targets) has adverse impact on service quality, productivity, and budget overruns and arrears. In the mission sector the state-owned enterprises run structural deficits even after subvention, have high debts, and struggle to make payments to suppliers.⁵⁴ The mission hospitals⁵⁵ account for 28 percent of the total sector arrears. Chronic data, information and analytical skill gaps hamper effective service organization and decisions on how to deliver more for the funding that hospitals receive.

Box 2.1. A Tool for Change - Block Contracting to Link Budget to Outcomes

- **How hospitals are financed can influence the quality and responsiveness of their services.** Under the current historic line item budgeting, funding is not associated with the activity, quality, and efficiency of service production and the risks are shouldered primarily by the fundholder. Increasing demands by providers for budget do not lead to commensurate improvements in services and health outcomes.
- **In Eswatini, block contracting can be the first step toward strategic purchasing⁵⁶.** Under a basic block contract, blocks of objectives (activity, quality, efficiency, and strategic)⁵⁷ are agreed between the purchaser and the provider and the provider needs to achieve the agreed objectives to receive funding. Under historic line item budgeting, the funds are released irrespective of service and performance targets. Block contracting transfers the risk to the hospital and challenges its service quality, patient-centered culture, and staff qualification to have positive effect on clients. It is an entry point to link funding to health services because it is a progressively adaptable provider payment method that can increase in complexity over time as data, management, and purchasing capacity are strengthened. Introducing block contracting is realistic, affordable, and can serve as an important applied training ground to strengthen purchaser and provider capacity in contract-based service delivery.
- **Block contracting is an important instrument for strategy implementation because the key objectives of the hospital strategy would be captured in the blocks of objectives with defined measurable targets.** The steps of block contracting are (a) model characterization, (b) proposal of objectives, (c) purchaser-

⁵³ For cases that cannot be managed in public facilities due to capacity constraints, the Phalala Referral Fund and the Civil Servant Medical Referral Scheme had been established to provide financial support to access specialist care in local private hospitals or outside the Kingdom.

⁵⁴ IMF. 2019. Eswatini: Exploring Options for Expenditure Rationalization. Technical Report.

⁵⁵ RFMH and Good Shepherd Hospital)

⁵⁶ Strategic purchasing is funding providers based on information about populations and providers to achieve performance goals

⁵⁷ Targets are defined between a min-maximum value. The achievement of the hospital is derived from a weighted score of results across the blocks.



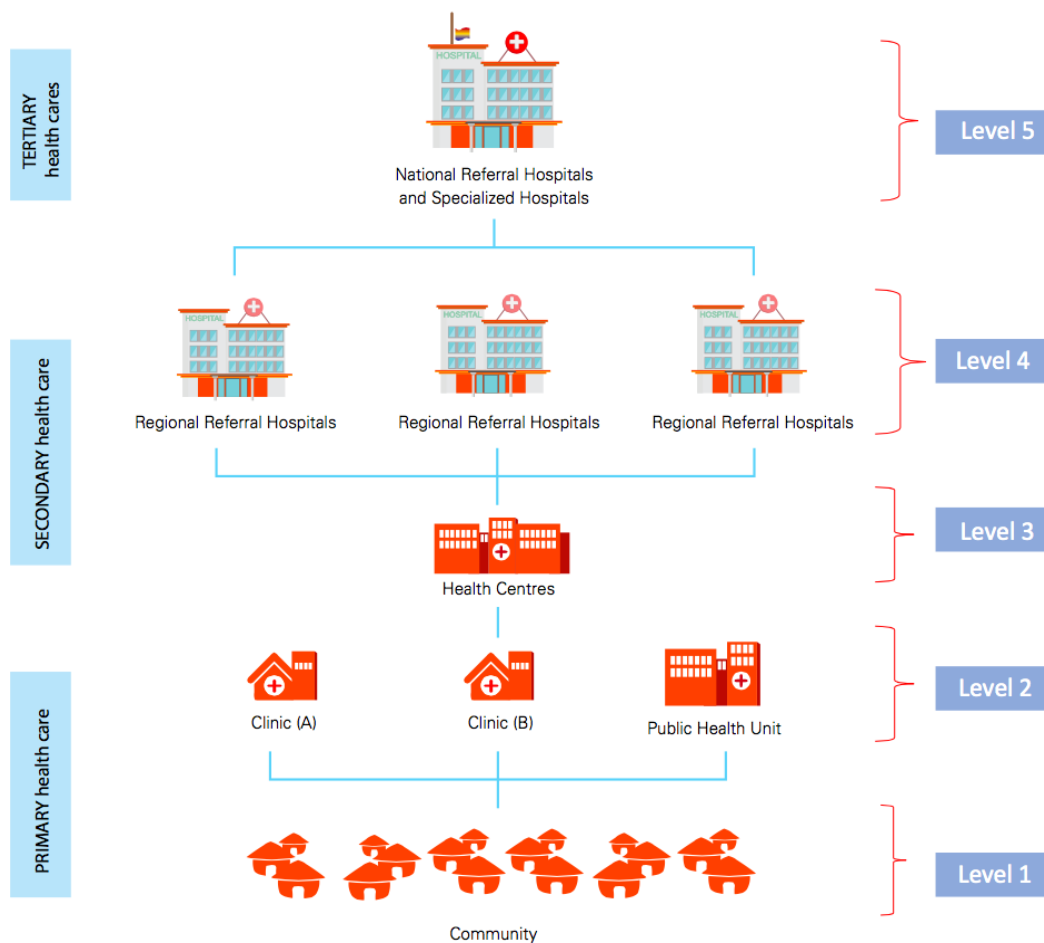
provider dialogue, (d) signing of contract, and (e) contract management and monitoring. Developing a block contract, therefore, would require technical support to define the objectives and targets and strengthen contract writing, negotiation and management skills.⁵⁸

- **Although a simple instrument, block contracting can be an important tool for change, an opportunity to hone skills to prepare the MOH or other agents to apply purchasing, over time with increasing complexity and at an increasing scale.**

⁵⁸ The support would cover capacity strengthening related to the contracting steps: (1) technical design/authoring; (2) negotiation; (3) approval/signing; (4) execution; (5) obligations management and monitoring; (6) contract amendment; (7) audit and reporting; and (8) renewal.



Figure 2.1. Levels of Care



Stewardship

MOH. Presides over national health policy making and stewardship for service delivery.

Regional Health Management Team is a cross-over point within the health system, where national policies and strategies are converted into action, local demands are aggregated and incorporated into action plans, and where horizontal relationships are established with other agencies and other sectors covering the same geographical area.

Levels of care

- **Tertiary care.** Curative and rehabilitative services through the *national referral hospital* Mbabane Government Hospital (MGH), the TB Hospital and the National Psychiatric Center (NPC) (Level 5).
- **Secondary health care.** *Health centers* providing preventive and curative in-patient services through a team of doctors, nurses and others (Level 3), and *regional hospitals* (4) dispensing specialized services (Level 4).
- **Primary health care.** Preventive and promotive services in *communities* (Level 1) and *clinics* managed by nurses providing curative and preventive services (Level 2).

Sources: MOH SARA 2017/18; PEPFAR 2016



Figure 2.2. Harmonized Approach to Investing in Human Capital in Eswatini

